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OECD ECONOMIC SURVEYS

1999

SPECIAL FEATURE
The health care system

BELGIUM

OECD ECONOMIC SURVEYS

1998-1999

BELGIUM

ORGANISATION FOR ECONOMIC CO-OPERATION AND DEVELOPMENT

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The previous Survey of Belgium and Luxembourg was issued in April 1997.

BASIC STATISTICS OF BELGIUM

THE LAND (1996)

Area (1 000 km^2)	30.5
Agricultural area (1 000 km^2)	13.4

Major urban areas (thousand inhabitants):	
Brussels	948.1
Antwerp	933.0
Liège	589.9
Ghent	491.7

THE PEOPLE (1997)

Population (thousands)	10 181
Inhabitants per km^2 (1995)	332
Net natural increase	11 000
Net migration (1995)	13 400

Total labour force (thousands)	4 320
Total domestic employment (thousands):	3 710
Agriculture	84
Industry and construction	942
Other	2 684

PRODUCTION (1997)

Gross domestic product (billion BF)	8 676
Gross domestic product per head (US$)	23 831
Gros fixed investment:	
Per cent of GDP	17.8
Pae head (US$)	4 230

Gross domestic product by origin, at market prices (per cent):	
Agriculture	2.0
Industry and energy	26.1
Construction	5.0
Other	66.9

THE GOVERNMENT (1997)

General government (per cent of GDP):	
Current expenditure	50.5
Current revenue	50.1
Gross debt	121.8

Composition of the House of Representatives (number of seats):	
Socialists	41
Christian-socials	41
Liberals	39
Others	29
Total	150

Last election: 21.5.1995

FOREIGN TRADE (1997)

Exports of goods and services (per cent of GDP)	72.9
Main exports (per cent of total), SITC (BLEU):	
Iron and steel products (67 + 68)	7.5
Chemical products (5)	18.0
Machinery and equipment (71 to 77)	13.2
Textile products (65)	4.2
Transport equipment (78 + 79)	15.1

Imports of goods and services (per cent of GDP)	68.4
Main imports (per cent of total), SITC (BLEU):	
Iron and steel products (67 + 68)	4.9
Chemical products (5)	15.2
Machinery and equipment (71 to 77)	17.2
Textile products (65)	2.6
Transport equipment (78 + 79)	12.6
Energy (3)	7.1

THE CURRENCY

Monetary unit: Belgian franc

Currency units per US$, average of daily figures:	
Year 1997	35.7584
November 1998	34.7042

Note: An international comparison of certain basic statistics is given in an annex table.

BELGIUM

Assessment and recommendations

Growth has been robust

The Belgian economy has performed fairly well over the past couple of years, with buoyant growth and low inflation. After a slowdown in 1996, real GDP growth picked up in line with developments in neighbouring countries and, at 3 per cent in 1997 and probably only slightly less in 1998, has been somewhat stronger than projected at the time of the previous EDRC examination of Belgium in January 1997. As business fixed investment has remained buoyant and private consumption and residential investment have strengthened, the driving force of the expansion has progressively shifted from net exports to domestic demand. The revival of private consumption has reflected not only stronger job creation and an acceleration in disposable personal income, but also a fall in the saving ratio, as consumer confidence has rebounded, possibly as a result of remarkable progress in fiscal consolidation and better prospects in the labour market.

Inflation is subdued and the current-account surplus large

Unlike in previous cycles, the wage-price performance has remained relatively good during this upswing. The end of the 1995-96 real wage freeze has been followed by only a modest acceleration in wage increases and compensation per employee. Several factors, both domestic and foreign, seem to have contributed to this result, including: the introduction of the law on employment and competitiveness which limits, on an *ex ante* basis, the maximum increase in compensation per employee in the private sector to the expected weighted average increase in the three reference countries, *i.e.* Germany, France and the Netherlands; the credibility of monetary policy; a declining but still significant output gap, estimated by the OECD Secretariat to be

around 1¼ per cent in 1998; and very subdued import prices as a result of the East Asian crisis and the fall in oil prices. CPI inflation has recently been at around 1 per cent. The current-account surplus has widened to nearly 5 per cent of GDP, one of the largest in the OECD area.

But unemployment, in all its forms, remains high

Labour market conditions have improved. This has been the result of cyclical factors, higher job content of growth, and administrative measures. The latter have resulted in an important shift of older unemployed from the normal unemployment benefit programme to special programmes which are not included in traditional measures of unemployment and the labour force, and they have therefore accelerated the decline in joblessness. The standardised unemployment rate has fallen by more than a full percentage point over the past couple of years, to 8.9 per cent in August 1998 – a rate below the EU average but well above the OECD average and still high by historical standards. The claimant-based unemployment rate (national definition) has followed a similar path, falling to 12.5 per cent in October 1998 – a level that is still higher than the structural rate estimated by the OECD Secretariat. Since the beginning of 1996, the decline in registered unemployed has been due in part to an increase in older unemployed not claiming normal unemployment benefits. The latest available data show, however, that over the past 12 months the total of registered unemployed and older unemployed has dropped by 5 per cent. A broader concept of labour underutilisation, which adds the older unemployed and all other people in special unemployment-related programmes to unemployment as usually defined, shows that the rate of "broad unemployment" fell from 25 per cent of the broad labour force in 1994 to 24 per cent in 1996 and 23 per cent in 1998.

The outlook is relatively favourable,...

Despite concern about the global economy, the outlook for Belgium remains relatively favourable. Given the limited importance of trade and financial links with emerging markets, the direct impact of problems in those areas on Belgian exports will be small. More generally, the impact of weaker export markets outside Europe should be partly cushioned by continued vigour in the euro area, including in Germany and France. The appreciation of the Belgian franc

in effective terms included in the projection erases only partially the 1996-97 depreciation; and, with a cost-price performance broadly in line with that of foreign competitors, Belgian firms are expected to remain internationally competitive. On the domestic side, after being contractionary for several years, fiscal policy is now likely to be broadly neutral, and no major tightening of monetary conditions is expected over the projection period. Finally, there are no major constraints on the acquired strength of domestic demand as the expansion remains remarkably free of tensions, the output gap is still significant, and the low employment rate provides additional leeway, especially if further structural measures enhancing employment are taken.

... although, due to the global financial crisis, the downside risk and uncertainty are significant

All considered, real GDP growth is projected to slow moderately – to around 2¼ per cent in both 1999 and 2000 – with a markedly lower rate of growth of exports. Private consumption could lose some of its recent buoyancy, but it will continue to be underpinned by the strength of disposable personal income. Similarly, while decelerating somewhat, business fixed investment is expected to remain relatively dynamic as a result of the high rate of capacity utilisation, good profits, and low interest rates. With economic growth somewhat above the potential growth rate, the output gap would partly disappear and the unemployment rate approach the estimated structural rate. Due to the restraining effect of the law on employment and competitiveness and subdued import prices, serious inflationary pressures are unlikely to arise. The major risks and uncertainties concern the international environment. If the global financial crisis were to worsen further with the dollar falling well below the level assumed in the projection, the effects – both direct and indirect – on the Belgian economy could be sizeable, mainly because of weaker net exports not only to emerging markets and the United States but also to neighbouring countries, and slack business investment as a result of falling orders and a drop in firms' confidence. On the other hand, if international financial markets were to settle down and no further decisive measures were taken to increase the potential growth rate of the Belgian economy, the balance of risk on inflation would appear to be on the upside, as improved labour market conditions might induce

households to reduce their saving ratio and spend more than projected.

Appropriate economic policies have qualified Belgium for EMU

The overriding goal of the authorities has long been to allow Belgium to participate in Economic and Monetary Union from its inception. This goal has been achieved largely thanks to a sound monetary policy and a strict fiscal policy which have interacted positively. The policy of closely linking the exchange rate of the Belgian franc with the Deutschemark has been very successful, boosting confidence in the currency and progressively reducing interest differentials with Germany to nearly zero, thereby significantly facilitating the correction of the budget deficit. This, combined with a medium-term strategy of fiscal consolidation which has been steadfastly pursued even at times of unfavourable conjunctural conditions, has resulted in a fall in the general government deficit from around 7 per cent of GDP in the early 1990s to 2 per cent in 1997, well within the Maastricht criterion; and the cyclically-adjusted deficit is estimated to have fallen from over 7 per cent of potential GDP to less than 1 per cent – one of the better performances in the OECD area. At the same time, the public debt-to-GDP ratio, although remaining extremely high, has declined from around 135 per cent in 1993 to 122 per cent in 1997, reflecting not only progress in fiscal consolidation, but also privatisation and asset management operations – including capital gains on official gold sales.

But the debt-to-GDP ratio must be reduced rapidly and the "quality" of public expenditure improved

Although Belgium has recorded some remarkable macroeconomic achievements over the past few years, there are a few major unresolved problems: the debt-to-GDP ratio is still very high; the composition of public expenditure is unbalanced; and structural reform remains insufficient, with a large number of persons on welfare. A rapid reduction in the debt-to-GDP ratio is a priority of the Belgian authorities. They are committed to keeping the primary surplus (*i.e.* excluding interest payments) at around 6 per cent of GDP over the medium term. This is welcome, since it could be expected to bring the overall general government budget into surplus already in a few years' time and reduce the debt/GDP ratio to 60 per cent by 2015 or earlier. Given that Belgium's overall deficit is already less than 2 per cent

of GDP, this commitment might be more stringent than the EU's Stability and Growth Pact. But, in order to avoid having to take corrective budgetary measures in an economic downturn, it would be appropriate to aim for a somewhat higher primary surplus when the economy is overheating. Another fiscal challenge is the need to improve the "quality" of public expenditure. The burden of fiscal consolidation having fallen disproportionately on public investment, this item has averaged 1½ per cent of GDP over the past decade and is among the lowest in the OECD area; on the other hand, social expenditure has remained above the EU average and the OECD average. To prevent the physical infrastructure from deteriorating, and possibly to improve it, it would seem desirable to increase public investment while at the same time restraining social expenditure. Hence, there is a need to reduce all passive spending on unemployment, to accelerate the reform of public sector pensions, and to keep health expenditure under control even if the "norm" limiting its annual growth in real terms to 1.5 per cent is dropped (see below).

To boost non-inflationary growth and reduce the large number of persons on welfare, the pace of structural reform must be stepped up

Due to rigidities in the labour and product markets, there is a risk that inflationary pressures might emerge while unemployment is still high. Stepping up structural reform would simultaneously increase non-inflationary potential growth, expand job creation and reduce the large number of persons on welfare. But progress in structural reform has been relatively slow in the past couple of years, and a number of important policy recommendations included in the OECD Jobs Strategy have not been considered. EMU and fiscal consolidation have been at the top of the policy agenda, and in the recent period the government has focused its efforts on the preparation of the EU National Action Plan for Employment. Within this framework, the Belgian authorities have continued to emphasise cuts in employers' social security contributions – focused on low-skilled workers – combined with "active measures" such as training and guidance. Employers' contributions are to be cut by a total of over BF 100 billion over the next six years, representing a reduction in the average labour cost of 3.4 per cent. As a counterpart, the social partners are expected to take measures to improve training and enhance employment; and the

plan d'accompagnement des chômeurs, which is Belgium's main initiative in the area of active labour market policies, is to be strengthened. Several other important measures have been taken, as the authorities have pursued their efforts to improve labour market outcomes while preserving social peace and preventing the emergence of a class of "working poor". At the same time, they acknowledge that technological progress, globalisation and EMU will require more adaptable labour and product markets, more regulatory reform, and more reliance on market forces.

Progress has been slow in increasing wage flexibility...

Despite the end of the 1995-96 wage freeze, the increase in wages has remained modest. But this very welcome wage moderation at the macroeconomic level has been achieved in the context of the law on employment and competitiveness, and may have brought with it potentially large harmful long-term effects arising from rigidities introduced by this law at the microeconomic level. The law should be phased out before these slow-building and difficult to detect microeconomic effects undermine potential growth. To prepare for this and avoid jeopardising wage moderation at the macroeconomic level, the focus of attention of employment policy should be put not only on reducing non-wage labour costs but also on freeing the wage formation process, especially at the low-wage end of the labour market. Over the longer term, the preferable solution to the problem of employing low-skilled workers is to raise their productivity through education, training, and life-long learning towards levels consistent with socially acceptable wages. Over the short to medium term, however, the creation, through normal market forces, of an important number of permanent low-skilled jobs as well as more entry-level jobs for the young may require wages and labour costs to fall to match the level of productivity of workers currently unemployable. As noted in the 1997 *Survey*, several provisional measures could facilitate the transition to a more flexible wage formation process, allowing downward wage adjustments at the microeconomic level and a widening of the wage structure: the social partners when negotiating collective agreements should lower the minimum sectoral wage to the statutory minimum ("*salaire minimum interprofessionnel*"); greater use could be made of "opening clauses" allowing firms, under

certain circumstances, to negotiate with their work force to pay below the minimum set in collective contracts at the sectoral level; and existing exceptions from the statutory minimum wage could be generalised to employers hiring various target groups – such as older persons, youngsters, or long-term unemployed. As mentioned below, these measures to increase employment opportunities for low-skilled workers should be accompanied by in-work benefits to maintain the income of the workers involved. More generally, employees' power in wage determination would be reduced by easing job protection rules and discontinuing the practice of administrative extension of collective agreements.

... and more needs to be done to strengthen incentives to work...

As little progress has been recorded in reforming unemployment and related benefit systems, most of the recommendations presented in the 1997 *Survey* remain valid: the duration of earnings-related unemployment benefits should be reduced, while co-operation and co-ordination with regard to availability and job-search controls between the federal body administering benefits and the regional placement services should be tightened. But, above all, benefit sanctions in case of refusal of suitable work should be applied much more strictly. The age limit for early retirement – currently at 58 – should be increased and ways should be explored to tighten or phase out special unemployment-related programmes – such as "*pré-pensions*" and "*chômeurs âgés*" – with the help of active policies, including, when appropriate, subsidised jobs.

... and improve active policies

With a view to facilitating the return of some of the older persons currently in special unemployment-related programmes to the active labour force, mandatory participation in the *plan d'accompagnement* should be broadened to include persons aged over 46; and to enhance its effectiveness, contact with job seekers should be more frequent and ways should be found to reduce problems arising from the special Belgian administrative set-up which, in this area, requires a distribution of responsibilities among the federal government, Communities, and Regions. Moreover, as many older inactive persons with obsolete skills are probably largely "unemployable" – *i.e.* can hardly be expected to find

a job regardless of their wage level – the development of special subsidised jobs (such as Smet jobs) may be the only way to return them to the active labour force, to perform work of social interest. Similar programmes for other target groups of persons who would otherwise also be "unemployable", such as youngsters with no job experience and lacking the required work attitude, have already been introduced on a rather limited scale. These programmes, which normally should provide only temporary jobs and be combined with training efforts, could be extended progressively.

Equity considerations should be addressed through a reform of taxation and social protection

The Belgian authorities attach great importance to keeping poverty at a low level. Over the longer term, the best way to avoid creating "working poor" is to improve the skills of the labour force. On-going efforts to reform the education system should thus be pursued. In the immediate future, however, since the wage cuts required to price low-skilled workers and people who have prematurely withdrawn from the labour force back into the market are deemed unacceptable for social and equity reasons, they should be combined with a reform of taxation and social protection based on the introduction of employment-conditional (or in-work) benefits to help assure a socially acceptable standard of living while preserving incentives to work. While equity considerations are important, job creation is even more so, since it is the key to a low rate of poverty based on work rather than passive income transfers, and hence more sustainable over the longer term and more conducive to social cohesion.

Improving competition in product markets would also help

For social and political reasons, increasing wage flexibility and redressing incentives to work has proved a difficult and slow process. To raise employment opportunities and ease acceptance of reforms, policy efforts should also concentrate on improving competition in product markets, especially in the service sector. For instance, entry conditions in the distribution sector should be liberalised since the current licensing procedure for the establishment of shops acts as an excessive barrier to entry, favouring incumbent shop owners without necessarily advancing societal objectives in such areas as urban planning. Moreover, rules on shop-opening hours should be relaxed further. Also, the

electricity sector needs to be put on a solid competitive foundation which will require a breaking up of Electrabel's strong dominance in the area of production and distribution, or, at the very least, introducing regulations to make the access to the transmission grid open and fair. These are two areas where the scope for improvement seems especially large, and where deregulation holds the promise, not only of improving consumer choice, but also of creating a significant number of jobs. This would facilitate the task of making the labour market more flexible which, in turn, is a crucial condition for reaping the full benefit of product market reform.

The health care system delivers good quality services at reasonable costs...

The Belgian health care system is dominated by a compulsory, comprehensive and universal public health insurance, with the management of the system essentially entrusted to non-profit organisations – *i.e.* mutual sickness funds or *mutualités* – and the role of the government limited to regulation and partial funding. At 7½ per cent of GDP, total health spending in 1997 was essentially the same as the European and OECD averages; and, on the basis of various indicators, it would seem that the overall health status of the Belgian population has improved significantly over the past decade, and is about average compared with that of other Member countries. Finally, broadly speaking, Belgium does not seem to be confronted with a problem of waiting lists, either for access to medical practitioners, or for hospital inpatient care.

... but it suffers from structural problems

The combination of comprehensive solidarity-based insurance, modest co-payments, a largely fee-for-service system, and independent medical practice, while very attractive for patients and practitioners, creates a weak or distorted pattern of incentives which is not sufficiently countered by government control and self regulation by the medical profession. This makes the system vulnerable to abuse and inefficiency, with an over-supply of doctors, dentists, physiotherapists and hospital beds, resulting in unnecessary medical acts. These structural shortcomings may help explain that it is in the health sector that public spending has risen most rapidly over the past 10 to 15 years; and that to restrain its growth the authorities had to move

progressively away from a fee-for-service system and introduce, first, fixed budgets or *forfaits* for specific medical acts, and then, a strict global "growth norm", limiting the annual rate of growth of health spending to 1.5 per cent in real terms, to be achieved in particular by greater use of lump-sum financing. But this "norm" was imposed as a temporary measure. Without it, the trend growth of health expenditure is likely to remain strong as a result of the introduction of new expensive medical technology and the growing impact of population ageing; and, owing to the commitment to pursue fiscal consolidation, the need to avoid increases in non-wage labour costs, and the authorities' reluctance to increase co-payments, there is little scope for boosting revenue. To maintain the system will entail continuing with structural reforms.

The Belgian authorities are taking corrective measures, but they may not be sufficient...

Given the special characteristics of health care – notably the difficulty that consumers have in making informed choices – the authorities want to reform the system within the existing framework of largely non-market arrangements, relying on *concertation* among the major players under the supervision and ultimate management of the government. The stated aim is to ensure that the Belgian health care system will continue to reconcile the principles of solidarity, quality of care, and freedom of choice at a reasonable cost (compared with other OECD countries). Some of the measures being introduced – notably a wider use of *forfaits*, reliance on "peer reviews", and the introduction of a central medical file ("*dossier médical centralisé*") backed up by comprehensive medical data banks – seem promising. Through a reduction of unnecessary medical acts, they could lead to significant savings. Measures have also been taken to contain the supply of health care. Other measures – such as better information and the *responsabilisation* of all parties – may also be tried but, unless they are combined with concrete guidelines or cost/price incentives, can hardly be expected to play a major role. Whether the current approach, as a whole, will be sufficient, over the longer term, to broadly equate expenditure and revenue will depend, among other factors, on difficult-to-quantify technical and medical considerations – such as the scope for a wider use of *forfaits* for specific medical acts, and the related potential savings. It is also

uncertain whether the Belgian system of *concertation*, which worked reasonably well in the past when health expenditure was being allowed to grow rapidly, can be relied upon to curb expenditure strongly and allocate it efficiently among competing claims.

... and may have to be complemented by more market-oriented initiatives...

If the choice were made to introduce more market-oriented initiatives these should focus on redressing the pattern of incentives, while taking care to avoid excessive weakening of the principle of equity or equality of treatment in the public health care system. Of course, decisions on the efficiency/equity trade-off require value judgements which ultimately belong to the socio-political realm. Nonetheless, if the sustainability of the Belgian health care system is in doubt and difficult decisions need to be taken to reduce consumption, more appropriate incentives would significantly facilitate the adjustment. Moreover, insofar as risk-based private insurance and *forfaits* already exist in the Belgian health care system, a further reduction of the set of medical services covered by the compulsory health insurance scheme (with greater latitude for additional coverage by *mutualités* or private insurance), combined with the introduction of capitation payments to curb the growth of spending under the existing fee-for-service system, would be more a question of degree than of principle.

... including a reassessment of the role of for-profit insurance companies and the mutualités, as well as stepping up preventive medicine

To allow private for-profit insurers to provide an efficient complement to public compulsory insurance would require a level playing field in the provision of health care insurance, a situation which currently does not exist since the *mutualités*, which also offer supplementary health insurance, are not subject to various taxes paid by private insurance companies. Moreover, the role of *mutualités* should be reconsidered: the provision of medical care might be more efficient if they were allowed to compete on the basis of contribution rates – possibly with no right to reject applicants, in order to avoid "cream skimming" – and if they were prevented from owning or controlling hospitals and pharmacies to avoid conflicts of interest. Finally, preventive medicine should be stepped up, drawing in particular on the results of public health surveys which should be conducted on a regular basis.

Summing up

The macroeconomic performance of the Belgian economy has, in many respects, been remarkable over the past couple of years: growth has picked up, inflation has remained low, the current-account surplus has widened, and employment has increased. Moreover, fiscal consolidation has continued at a sustained pace, and Belgium has qualified for EMU – the overriding priority of the Belgian authorities. But these achievements should not lead to complacency: the debt-to-GDP ratio is still extremely high and must be reduced rapidly; the "quality" of public spending needs to be improved, partly because the burden of fiscal consolidation has in the past fallen disproportionately on public investment; and, although the economy is moving closer to its potential, the number of persons on welfare remains large. Within the Belgian framework of social consensus and equitable income distribution, the focus of attention of labour policy should be on returning low-skilled workers to the active labour force through better skills and qualifications, stronger incentives to work, and a more flexible wage formation process to price them back into the market. Progress in this direction would be significantly enhanced by more competitive and dynamic product markets: especially in the service sector the scope for reform would seem considerable. The favourable conjunctural situation provides a window of opportunity to step up the pace of structural reform, which would lay the ground for continuing strong non-inflationary growth and for a sustained increase in employment opportunities.

I. Recent trends and prospects

Overview

The macroeconomic performance of the Belgian economy has been rather good in 1997 and 1998. Economic activity has generally been robust, with few tensions or imbalances – the main exception being the labour market, where there has, nonetheless, been a noticeable improvement. The driving force of the expansion has progressively shifted from net exports to domestic demand, and this trend is expected to continue in the near future. Real GDP growth may have been nearly 3 per cent in 1998, and is projected at around 2¼ per cent in both 1999 and 2000. Job creation has already accelerated, and the unemployment rate has begun to decline but remains higher than the structural rate (NAWRU). Over the coming two years, unemployment, although falling, will remain high, and a large number of persons will continue to be inactive in special welfare programmes, such as those for early retirement and older unemployed. Consumer price inflation has declined markedly and, partly as a result of subdued import prices, is set to rise only moderately as the expansion matures. At 4¾ per cent of GDP, the current-account surplus is well above the EU average and one of the largest in the OECD area.

Robust economic activity

The expansion gained momentum in 1997 and the first half of 1998, but the conjunctural indicators of the National Bank of Belgium and other provisional data point to a significant slowdown in economic activity in the second half of the year, with weaker exports. Nonetheless, real GDP growth, which reached 3 per cent in 1997, may have been nearly as strong in 1998 (Table 1). With a potential growth rate estimated by the OECD Secretariat at around 2 per cent, the output gap has progressively shrunk, to an estimated 1¼ per cent in 1998. The upturn was initially fuelled by external demand. A sizeable depreciation of the Belgian franc in effective terms, coupled with a relatively good cost-price performance and strengthening activity in neighbouring countries, led to a surge in foreign

Table 1. **Demand and output: recent trends and projections**

Annual percentage change, 1990 prices

	1995 current prices		1997	1998	1999	2000
	BF billion	Per cent of GDP				
A. Demand and output						
Private consumption	5 071	62.9	2.1	2.8	2.1	2.2
Government consumption	1 182	14.6	0.8	1.1	1.6	1.6
Gross fixed investment	1 430	17.7	5.4	4.9	3.8	3.4
Final domestic demand	7 683	95.2	2.5	2.9	2.4	2.4
Stockbuilding[1]	33	0.4	–0.3	0.2	0.1	0.0
Total domestic demand	7 716	95.6	2.2	3.2	2.5	2.4
Exports of goods and services	5 395	66.9	7.1	6.1	5.3	5.4
Imports of goods and services	5 042	62.5	6.3	6.6	5.8	5.7
Foreign balance[1]	352	4.4	0.9	–0.1	–0.2	–0.1
GDP at constant prices	..	..	3.0	2.9	2.3	2.3
GDP price deflator	..	..	1.4	1.3	1.4	1.4
GDP at current prices	8 068	100.0	4.5	4.2	3.7	3.7
B. Memorandum items:						
Private consumption deflator	..	..	1.8	1.0	1.2	1.4
Private compensation per employee	..	..	2.1	2.1	2.2	2.0
Total employment	..	..	0.5	1.4	0.8	0.7
Unemployment rate (per cent)[2]	..	..	12.5	11.7	11.5	11.3
Breakdown of gross fixed investment						
Public sector	116	1.4	19.2	6.6	2.0	2.0
Private sector residential	437	5.4	4.9	3.6	3.5	3.0
Private sector non-residential	877	10.9	4.2	5.3	4.1	3.8
Interest rates (per cent)						
Short-term	..	..	3.4	3.6	3.0	3.1
Long-term	..	..	5.7	4.8	4.2	4.4
Current balance (per cent of GDP)	..	..	4.7	4.8	4.7	4.7

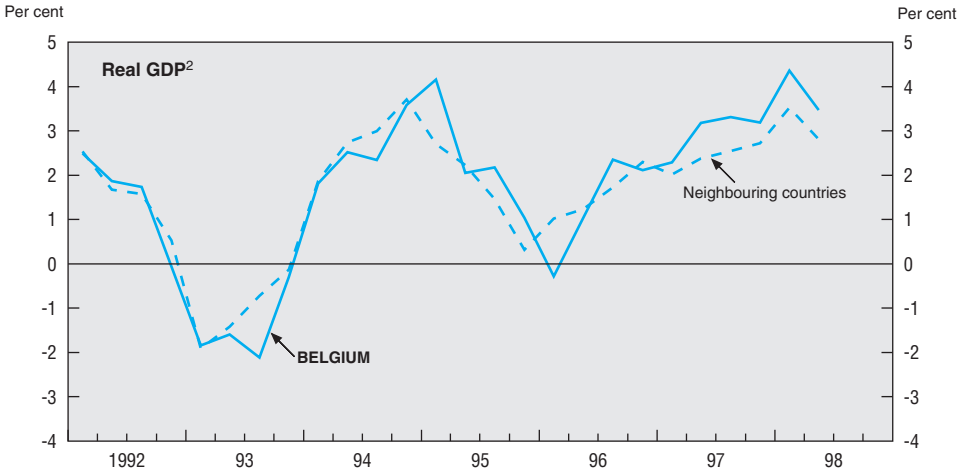
1. Contribution to growth of GDP.
2. National definition.
Source: OECD Secretariat.

orders, particularly for intermediate goods, which typically are in strong demand in the initial phase of a cyclical upturn. Since late 1997, however, foreign orders have weakened, while business fixed investment and private consumption have gathered strength, taking over as the driving force of the expansion.

Since the cyclical trough at the beginning of 1993, the strength of the Belgian economic expansion has been very similar to that in neighbouring countries, and in particular Germany, France and the Netherlands (Figure 1). On the other hand, its composition has been somewhat different. While, until recently, private consumption in Germany and France has remained hesitant, perhaps hindered by actual and prospective fiscal consolidation, in Belgium, it has strengthened, as the general government deficit has fallen well below the Maastricht ceiling, fears of further fiscal tightening have been largely allayed, and consumer confidence has surged (Chapter II). Nonetheless, private consumption and real GDP growth in Belgium have not been as rapid as in the Netherlands.

Business investment, although somewhat volatile, has been the most vigorous component of domestic demand in the current expansion, buoyed by improved profits, demand factors, and historically low interest rates (Figure 2). In addition, the rate of capacity utilisation in the manufacturing sector has reached unprecedented high levels.[1] But the overall increase in investment conceals substantial differences at the sectoral level, with investment in manufacturing losing much of its early dynamism (except in certain industries such as chemicals and textiles), and investment in services surging, particularly in banking. While investment has been strong, it has clearly fallen short of the exceptionally high level reached during the investment boom in the second half of the 1980s,

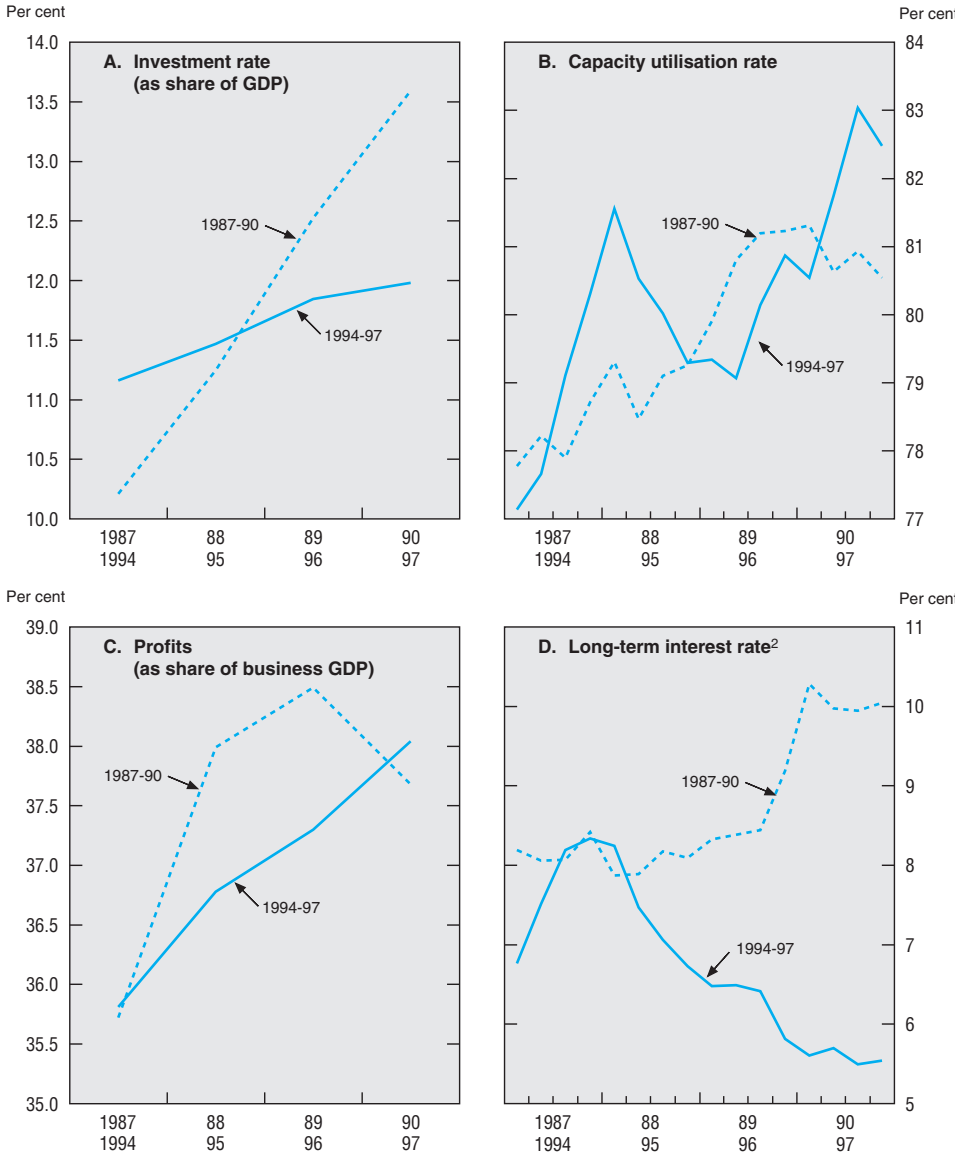
Figure 1. **The business cycle in Belgium and three neighbouring countries**[1]



1. Germany, France and Netherlands (weighted average).
2. Percentage changes from previous year, seasonally adjusted, volume.

Source: National Bank of Belgium and OECD Secretariat.

Figure 2. **Investment:[1] comparison of two business cycles**



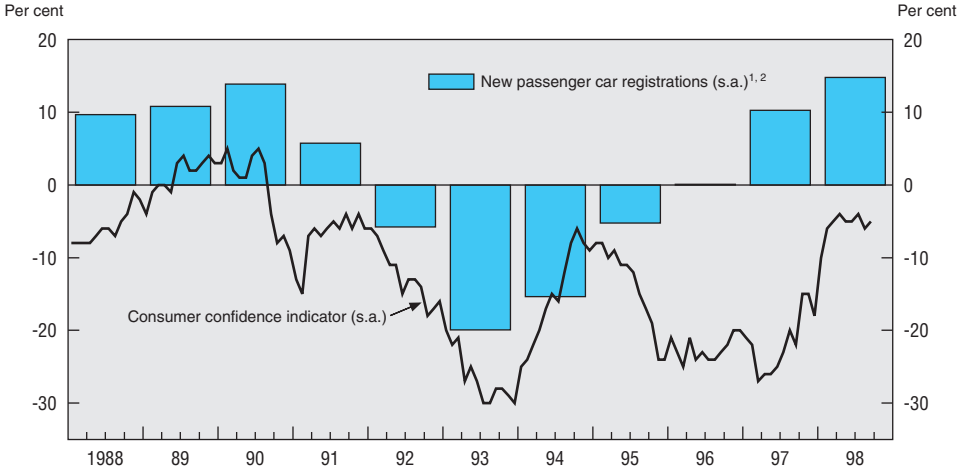
1. Business fixed investment.
2. Yield on government bonds (10-year).

Source: OECD, *Main Economic Indicators* and OECD Secretariat.

despite the fact that real interest rates are lower in this expansion than in the previous one, and capacity utilisation is higher. This performance may partly be explained by profitability which, while rising, is still below the record levels of the end of the 1980s (Figure 2C).[2]

Since the second half of 1997, the expansion has become more broadly based as private consumption and residential investment have picked up markedly. Consumption has apparently benefited from the slow but continuing improvement in labour market conditions as well as from progress in fiscal consolidation. Consumer confidence increased significantly following the presentation of the 1998 budget, as it became apparent that no further significant fiscal tightening was to be expected. This "feel good" factor, combined with substantial capital gains on financial assets and lower interest rates, has affected households' spending patterns. Car sales, one of the most cyclical components of private consumption, rose sharply in 1997, and remained strong afterwards – even allowing for the effect of the biennial Brussels' Motor Show (Figure 3). The saving ratio, which has been on a declining trend since reaching a high of 20½ per cent in 1993, fell to 15.2 per cent in 1997 and may have fallen further in 1998 – to slightly below 15 per cent.

Figure 3. **Consumer confidence and car registrations**



1. Average of monthly figures. Percentage change over two years.
2. 1998: Average of the first nine months.

Source: OECD, *Main Economic Indicators.*

With falling mortgage rates, better consumer confidence and improving labour market conditions, residential investment, after dipping in 1996, rebounded in 1997 and 1998. Housing construction has also been supported by a temporary reduction in VAT for some specific housing categories, which was introduced for 1996 and 1997, and was later extended to the first half of 1998.[3] Since builders speeded up construction to meet this deadline, residential investment is likely to have shown a corresponding lack of buoyancy in the second half of the year. Owing to fiscal consolidation, the volume of government consumption hardly increased in 1997 and 1998, and employment in the government sector was virtually stable. On the other hand, public investment, after falling sharply for a few years, rebounded by almost 20 per cent in 1997 and its growth seems to have remained strong in 1998, as sales of buildings were greatly reduced. Moreover, local authorities stepped up investment, taking advantage of their improved financial situation following the partial privatisation of the *Crédit communal*. Despite these increases, public investment – at only 1½ per cent of GDP – remained low both in historical terms and compared with other OECD countries.

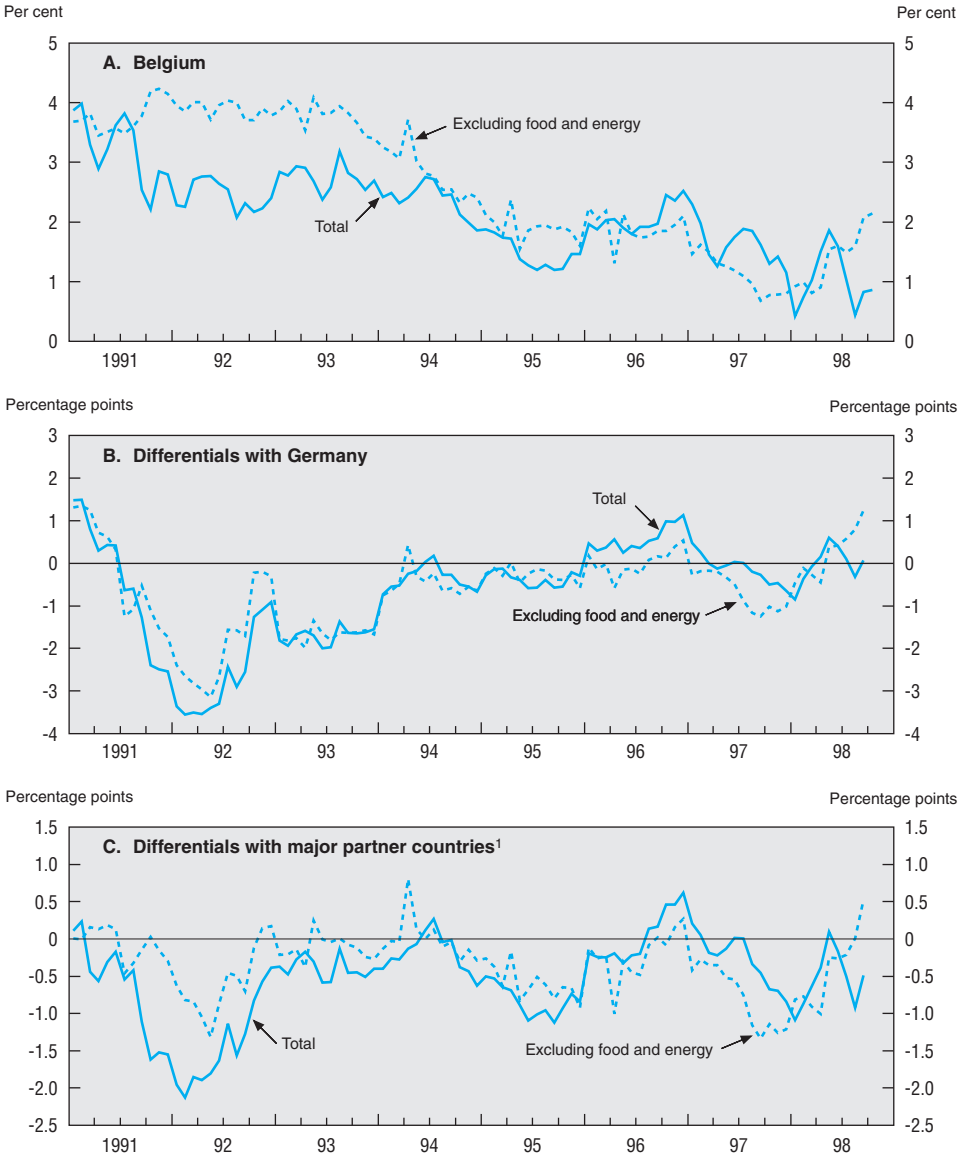
Export demand, which was the driving force of the expansion, started weakening in late 1997. This can be partly ascribed to the specialisation of the Belgian economy in the production of intermediate goods – a characteristic which becomes less supportive of growth as the expansion progresses. But it has also reflected the impact of the crisis in emerging markets on Belgian exports, a crisis of which the indirect impact, although still relatively small, has progressively grown. While overall exports have flagged, the growth of imports has accelerated, due to the strengthening of domestic demand. For 1997 as a whole the contribution of net exports to growth increased, to nearly 1 per cent of GDP; it seems, however, to have turned slightly negative in 1998.

Little or no inflationary pressures

An important feature of the current expansion is the good inflation performance. Whereas in previous cycles inflationary pressures built up during upturns, inflation is now under much better control. Several factors, both domestic and foreign, seem to have contributed to this result, including: the change in the index-linking mechanism in 1993-94, the 1995-96 real wage freeze and the subsequent law on employment and competitiveness; a declining but still significant output gap; very subdued import prices as a result of the fall in oil prices and the East Asian crisis; and the credibility of monetary policy.

The trend decline in consumer price inflation, which started at the beginning of the decade, has continued. After falling to a very low level in early 1998, inflation accelerated a little in the wake of a more rapid increase in food prices, before falling back to under 1 per cent in October (Figure 4, Panel A).[4] Increases in

Figure 4. **Consumer prices**
Change over 12 months



1. Weighted average, as a function of the geographical structure of the imports and exports of Belgium with Germany, France, Italy, the United kingdom and the Netherlands in 1991.

Source: OECD, *Main Economic Indicators* and *Foreign Trade Statistics*, Series C.

service prices and rents have continued to slow, the latter reflecting lower interest rates and subdued real estate prices. Deregulation, notably in the telecommunications sector, has also contributed to downward pressure on prices. However, excluding the impact of increases in indirect taxes on consumer prices – 0.5 per cent in 1996 and 0.2 per cent in 1997 – inflation has shown less pronounced variations; and underlying inflation – that is, excluding food and energy – has been on an upward trend since the second half of 1997, rising from less than 1 per cent to over 2 per cent in October 1998. While consumer price inflation has been broadly in line with that in Germany and somewhat lower than that of Belgium's major trading partners on average (Figure 4, Panels B and C), underlying inflation has been somewhat higher in Belgium than in other countries on average.

The 1995-96 real wage freeze has been followed by only a modest acceleration in the wage rate and in compensation per employee. To prepare for the transition, the government introduced in the summer of 1996 a law on employment and competitiveness. The law aims at limiting, on an *ex ante* basis, the maximum increase in compensation per employee in the private sector to the weighted average increase in the three reference countries, *i.e.* Germany, France and the Netherlands.[5] On this basis, the government set the maximum increase in compensation per employee (adjusted for hours worked) for the 1997-98 period at 6.1 per cent for the two years combined. In general, wage settlements and increases in compensation per employee seem to have remained below this legal limit, although in some sectors – such as the petro-chemical industry – they have

Table 2. **Labour costs in enterprises**[1]

Percentage changes compared with the previous year

	1991	1992	1993	1994	1995	1996	1997[2]
Gross wage per full-time employee	7.8	5.6	4.8	3.0	3.1	1.8	2.5
Increase resulting from:							
Indexation	3.5	2.7	2.6	1.4	1.5	1.4	1.5
Wage increases under collective agreements	2.3	2.3	0.9	1.4	0.0	0.0	0.8
Wage drift	1.8	0.6	1.3	0.2	1.6	0.3	0.2
Employers' social security contributions	0.3	0.1	0.1	–0.5	–0.5	–0.4	0.2
Compensation per full-time employee	8.1	5.7	4.9	2.5	2.6	1.4	2.7
Productivity per full-time employee	1.8	2.0	–0.2	3.1	1.8	0.3	3.0
Unit labour costs[3]	6.2	3.6	5.1	–0.6	0.8	1.1	–0.3

1. Private and public enterprises. All data are calculated without taking account of the possible influence of the statistical adjustments whereby the three national accounts' approaches are reconciled.
2. Estimates.
3. Per unit of value added, at constant prices.

Source: National Accounts Institute and National Bank of Belgium.

reportedly exceeded it. On average, both the wage rate and compensation per employee increased by a little over 2 per cent in 1997 – compared with 1 per cent in 1996. On a full-time basis, compensation per employee accelerated from 1.4 to 2.7 per cent, but as productivity also accelerated markedly, unit labour costs in the private sector decreased slightly – by 0.3 per cent in 1997, compared with an increase of 1.1 per cent in 1996 (Table 2). Hence, despite the end of the wage freeze, wage-cost developments have been broadly in line not only with those in the three reference countries, but also with those in Belgium's partner countries on average.

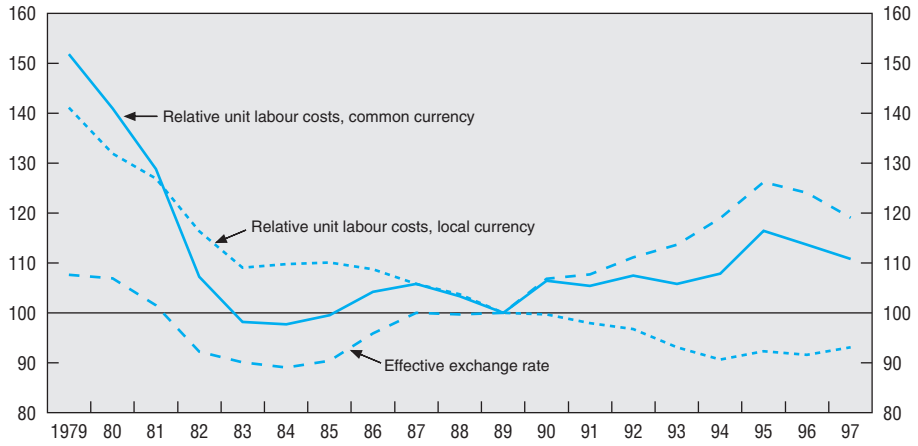
A large current-account surplus

Belgium's current-account surplus widened in 1997: at 4.7 per cent of GDP it was well above the EU average – 1.5 per cent of GDP – and the sixth largest in the OECD area. The increase in the surplus largely reflected a stronger trade balance: exports picked up in response to more buoyant economic conditions in EU countries – which account for some 75 per cent of Belgium's exports – and to a significant improvement in cost-price competitiveness. Unit labour costs in a common currency (in manufacturing) declined significantly in 1997 for the second year in a row, as the Belgian franc in effective terms continued to depreciate while, as just noted, the cost-price performance of Belgium was broadly similar to that of its trading partners (Figure 5). The impact of the East Asian crisis in 1997 was limited, given that Belgium's exports to these countries (including Japan), represent only some 5 per cent of total exports. Nonetheless, exports of goods to Southeast Asia and Japan fell by 15 per cent in the fourth quarter of 1997, and the growth of exports to China decelerated sharply.

While exports lost some buoyancy as 1997 progressed, imports strengthened as a result of the pick up in domestic demand, and especially in consumer durables. Earlier on, imports had been underpinned by the high import-content of exports and transit trade. Import prices, and particularly primary commodity prices, increased in 1997, owing to the decline of the Belgian franc in effective terms; however, export prices rose almost in parallel with import prices, reflecting the large share of re-exports and the typical passing-on of higher costs of primary imports. Moreover, towards the end of the year, energy prices fell sharply. As a result, the terms of trade worsened only slightly.

Among the other items of the current account, net investment income credits increased further in 1997, partly as a result of the continuing build-up of net claims on the rest of the world and the appreciation of the dollar and sterling – in which a large proportion of the stream of inward investment income is denominated.

Figure 5. **International cost competitiveness**[1]
Index 1989 = 100



1. In manufacturing.
Source: OECD Secretariat.

In 1998, largely as a result of a decline in energy prices, the terms of trade are expected to have improved somewhat. Hence, despite a growing adverse impact of the crisis in emerging market economies on the volume of Belgium's exports, the trade surplus may not have changed greatly. With the other items, on balance, showing little variation, the current-account surplus may have widened only slightly, to 4¾ per cent of GDP.

But high unemployment

Policy measures and problems in the labour market are discussed in Chapter III. This section briefly reviews recent developments.

The economic expansion has begun to improve labour market conditions in a significant way, although a precise assessment is hampered by several policy measures and administrative decisions – such as incentives for part-time work, direct job creation by the *Agences locales pour l'emploi* and, in particular, a shift of older unemployed to special programmes entailing their exclusion from the traditional indicators of the labour force and unemployment. Available indicators show that recruitment has picked up since early 1996; and since the beginning of 1997,

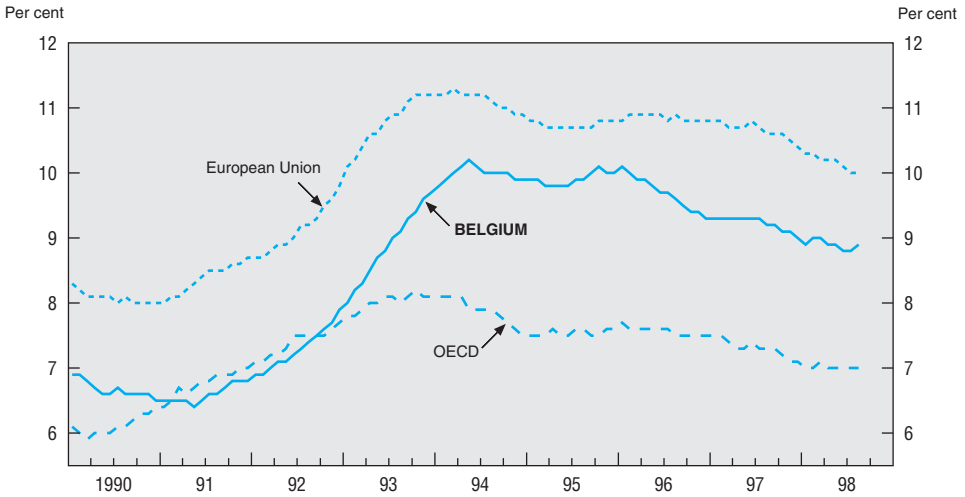
a growing proportion of enterprises have experienced shortages of skilled workers, and the number of unfilled vacancies has started to climb. Employment increased by ½ per cent in 1997, and may have increased at a much faster rate in 1998 – over 1 per cent. A special feature of this expansion has been the very rapid increase in temporary work: nearly 20 per cent in 1997 and the first half of 1998 (after 7 per cent in 1996 and 18½ per cent in 1995).[6] The most noticeable increase in employment has been amongst the under-25s and the over-50s.

Growth may have become more labour intensive, especially in terms of persons per output, as a result of wage moderation and cuts in employers' social security contributions which favour the substitution of labour for capital. Policy measures to encourage work sharing may also have gone in that direction – especially in the public sector. Part-time working has developed steadily and is moving closer to the European average. Private enterprises have shown little interest in government measures to promote collective reductions in hours worked.[7]

Unemployment has been on a declining trend since early 1996. The standardised unemployment rate has fallen by more than a full percentage point over the past couple of years – to 8.9 per cent in August 1998. While this rate is below the EU average, it remains historically high and well above the OECD average (Figure 6). The unemployment rate (national definition)[8] has followed a similar path, falling to 12.5 per cent in 1997 and to 11.7 per cent (partly estimated) in 1998. The NAWRU is estimated by the OECD Secretariat at around 11 per cent and at 8 per cent by the Belgian authorities. Caution is needed in interpreting the indicator because of calculation difficulties (mainly owing to the fact that the Belgian authorities have intervened frequently in the wage formation process), and even more so in a small and very open economy. Apparent regional differences in unemployment in Belgium – an unemployment rate of 7.7 per cent in Flanders, 17.2 per cent in Wallonia and 19 per cent in Brussels – mask major sub-regional divergencies. This and other indicators – such as the continuing high proportion of long-term unemployment and the disparities in unemployment rates according to gender and educational achievements or experience – suggest that unemployment remains largely structural.

The decline in recorded unemployment has, to some extent, reflected administrative measures, so that "broad unemployment" has evolved less favourably. For instance, since the decision, at the beginning of 1996, to ease the conditions under which older unemployed[9] may be exempted from job search and hence can shift from the normal unemployment programme to the special programme for older unemployed (*chômeurs âgés non demandeurs d'emploi*), the number of full-time unemployed receiving a benefit (*chômeurs complets indemnisés*) has decreased by some 76 000[10] but, over the same period, the number of older unemployed has increased by 60 000.[11] A broader concept of labour under-utilisation ("broad unemployment"), which adds people in special

Figure 6. **Unemployment rate:[1] an international comparison**



1. Standardised rate.
Source: OECD Secretariat.

unemployment-related programmes to unemployment as usually defined ("narrow unemployment"), shows that in this respect conditions changed only slightly in 1997 and 1998.[12, 13] As the decline in "narrow unemployment" and in "other unemployed" was partially offset by the increase in "people in special programmes" – particularly *chômeurs âgés* – "broad unemployment" barely edged down, still representing 22½ per cent of the broad labour force (Table 3).[14] All forms of unemployment and inactivity among working-age persons – regardless of whether they receive a social security benefit or not – are of course reflected in the overall rate of employment – the broadest indicator of the degree a country uses its potential labour force.[15] At 57 per cent in 1997, the employment rate in Belgium was one of the lowest in the OECD area (OECD, 1998), but the negative deviation from the OECD average has narrowed appreciably in the last few years, from 7.9 points in 1990 to 3.1 points in 1997.

A relatively favourable outlook

Several considerations suggest that, despite the turmoil in world financial markets, real GDP growth in Belgium should remain robust. First, given the

Table 3. **Unemployment and special programmes**[1]

Thousands

	1982	1990	1993	1994	1995	1996	1997	1998
I. Narrow unemployment (*population active en chômage*)	490	365	511	554	555	545	541	505
II. Other unemployed	80	230	214	178	159	131	112	103
Seasonal unemployed (*chômeurs temporaires*)	57	25	48	36	35	32	37	29
Part-time unemployed (*chômeurs à temps réduit involontaire*)	23	205	166	142	124	99	75	74
III. People in special programmes	163	430	420	405	395	420	428	447
Exempted from job search:								
For age reasons (*chômeurs âgés*)	0	72	75	74	74	95	114	131
For social reasons, training and studies (*interruption de chômage*)	0	51	46	42	41	43	33	28
Special leave (*interruption de carrière*)	0	49	58	54	51	52	56	67
Early retirement (*pré-pension*)	109	163	146	141	134	134	129	125
Direct job creation (*remise au travail*)	54	95	95	94	95	96	96	96
IV. Broad unemployment (I + II + III)	733	1 025	1 145	1 137	1 109	1 096	1 081	1 055
As a percentage of the broad labour force[2]	17.3	22.7	24.9	24.8	24.1	23.7	23.2	22.5
Memorandum items:								
V. Other unemployed and people in special programmes (II + III)	243	660	633	583	554	551	540	550
VI. Broad unemployment in full-time equivalent	596	835	944	954	909	915	925	902

1. Registration basis, end-June data.
2. Labour force + exempted from job search + special leave + early retirement.

Source: OECD Secretariat and Ministry of Employment and Labour.

limited importance of trade and financial links with emerging markets, the direct impact of problems in those areas on Belgium's exports, although still growing, may remain small. More generally, the impact of weaker export markets outside the euro area should partly be cushioned by a relative vigour in this area – Germany and France included. Also, the exchange rates used for the projection imply an appreciation of the Belgian franc in effective terms which does not fully erase the depreciation recorded in 1996-97. This, combined with a cost-price performance broadly in line with that of foreign competitors, should allow Belgian firms to remain internationally competitive. Moreover, after being contractionary for several years, fiscal policy is now likely to be broadly neutral, and no major

tightening of monetary conditions is expected in the euro zone over the projection period (Chapter II). Finally, although starting already four years ago, the expansion in Belgium remains remarkably free of tension, the output gap is still significant, and the low employment rate could provide additional leeway to boost non-inflationary growth, especially if further structural measures enhancing employment were taken (Chapter III).

Real GDP growth is projected to slow only moderately – to around 2¼ per cent in both 1999 and 2000. Weaker exports may slow economic activity somewhat although, given the openness of the economy and the consequent dampening effect on imports, the contribution of the foreign balance to growth may not change greatly and may remain only slightly negative. While decelerating, domestic demand should remain the driving force of the expansion. Business fixed investment is expected to continue to grow at a satisfactory rate as a result of the high rate of capacity utilisation, good profits, and low interest rates. Private consumption is also projected to stay relatively dynamic: in addition to the "feel good" factor related to progress in fiscal consolidation and improving labour market conditions, it will be supported by sustained growth in real disposable income.

With economic growth somewhat above the potential growth rate of the Belgian economy – estimated by the OECD Secretariat at around 2 per cent – the output gap should shrink further and nearly disappear in 2000; and the unemployment rate should continue to fall. Owing to a declining but still significant output gap, the restraining effect of the law on employment and competitiveness, and subdued import prices, serious inflationary pressures are unlikely to arise. The increase in compensation per employee and in the private consumption deflator may be only around 2 per cent and a little less than 1½ per cent, respectively, in 2000. The current-account surplus is projected to remain at around 4¾ per cent of GDP.

The major risks and uncertainties attaching to this projection concern the international environment and – at this stage – would seem to be on the downside. The Belgian economy could incur severe losses if the global financial crisis were to worsen further, with the dollar falling well below the level assumed in the projection. This would entail weaker net exports not only to emerging countries and the United States but also, and perhaps more importantly, to neighbouring countries, as economic growth in the euro zone would slow markedly. Also, business investment would slump as a result of falling orders and a drop in firms' confidence. On the other hand, if international financial markets were to settle down and, on the domestic side, no further decisive measures were taken to increase the potential growth rate of the Belgian economy, there could be some risk of inflationary pressure, as improving labour market conditions might induce households to reduce their saving ratio and spend more than projected.

II. Economic policies

The overriding goal of the Belgian authorities has long been to meet the Maastricht criteria and thereby to participate in the Economic and Monetary Union (EMU) from its inception. Through a medium-term strategy of fiscal consolidation, which has been steadily pursued even at times of unfavourable conjunctural conditions, the general government deficit has been reduced from around 7 per cent of GDP in 1992 to 2 per cent in 1997, well within the Maastricht criterion; and the cyclically-adjusted deficit has been reduced from over 7 per cent of potential GDP to less than 1 per cent, one of the better performances among OECD countries. At the same time, the public debt-to-GDP ratio, although remaining very high, has declined from around 135 per cent in 1993 to 122 per cent in 1997. On the monetary side, the policy of closely linking the exchange rate of the Belgian franc with the Deutschemark has been a remarkable success: it has contributed to boosting confidence in the currency and progressively reducing interest differentials with Germany to nearly zero, thereby significantly enhancing the process of fiscal consolidation. On the other hand, with EMU and fiscal consolidation at the top of the policy agenda, structural reform has been relatively slow in the past few years, and the scope for further progress in this area remains important, especially in the labour market and in product markets.

The following paragraphs, after briefly reviewing monetary policy and monetary conditions, consider the broad lines of budget policy and progress in fiscal consolidation. Structural measures in general are discussed in Chapter III, while initiatives related to the health care system in particular are considered in Chapter IV.

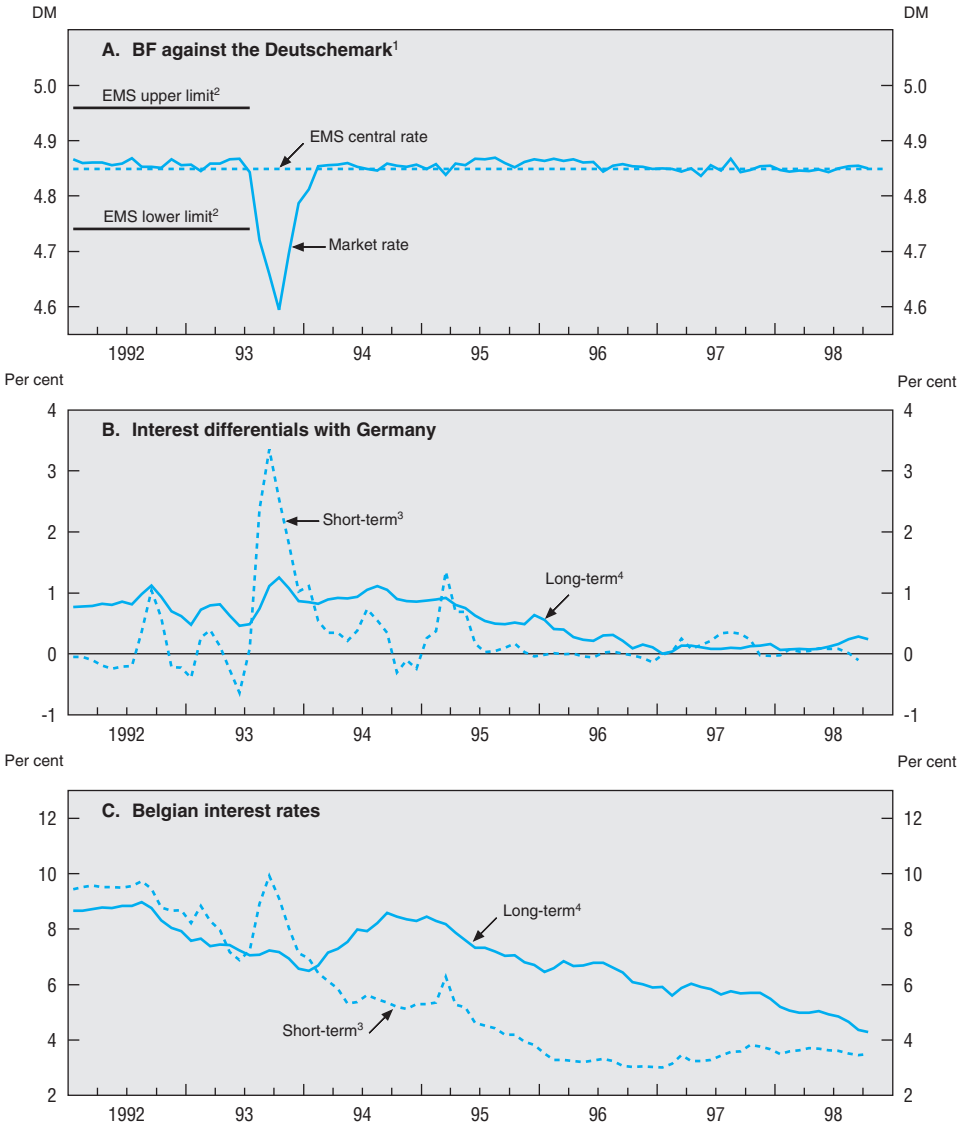
Monetary policy

Despite the recent international financial crisis, the period since early 1997 has been characterised by growing confidence in the timely introduction of the euro, continuing strength of the Belgian franc within the European Monetary System, and a pronounced trend decline in European long-term interest rates. In this relatively favourable environment, and since the sizeable current-account

surplus of the Belgian-Luxembourg Economic Union (BLEU)[16] has been broadly matched by spontaneous financial outflows, the intermediate goal of monetary policy – *i.e.* a stable exchange rate *vis-à-vis* the Deutschemark – has been relatively easy to achieve, without explicit policy measures (Figure 7, Panel A). Nonetheless, through its liquidity management, the National Bank of Belgium has occasionally supported – notably in February and March 1997, from July to October 1997 and from February to April 1998 – spontaneous but short-lived rises in Belgian money market rates above corresponding German rates. Despite these slight tensions, short-term interest rate differentials between the two countries have generally remained rather small, with rates edging up as in several other continental European countries (Figure 7, Panels B and C). In early October 1997, in the context of accelerating economic growth and after a marked rise of the exchange rate of the dollar, the National Bank of Belgium raised its interest rates at the same time as the central banks of Germany, France, the Netherlands, Austria and Denmark. In the view of the Bank, this represented a switch from a relatively expansionary stance of monetary policy to a more neutral position, in order to prevent any resurgence of inflation. On the other hand, Belgian long-term rates have trended down and somewhat faster than corresponding German rates, so that the differential has shrunk. The recent turmoil in international financial markets has had relatively little impact on Belgium, although the long-term interest differential with Germany – which had stabilised at around 10 basis points in the first half of 1998 – has widened anew, to around 30 basis points. Also, Belgian equity prices have recorded a significant downward correction, before erasing a part of their losses, broadly in line with other continental financial centres.

At less than 4½ per cent, the yield on Belgian government bonds is currently at an historically low level and one and a half percentage points lower than at end-1996. As the depreciation of the Belgian franc in effective terms recorded in 1997 has not been fully offset by its recent appreciation, overall monetary conditions would seem to have eased – even though, as noted, short-term interest rates have edged up and, as a result of a reduction in inflation, long-term interest rates have declined less in real than in nominal terms. The impact of such a monetary easing on the real economy, however, may be less clear-cut than in certain other countries. While in a small open economy like Belgium a depreciation of the nominal exchange rate in an environment of price stability, like the present one, can be expected to have a significant positive impact on net exports and growth, the impact of lower long-term rates is more uncertain and may even be negative. Owing to the very large (direct and indirect) holdings of government bonds by households, a decline in the yield on these securities, in real terms, represents a fall in disposable income which may negatively affect private consumption and household spending in general.[17] All in all, monetary conditions seem broadly appropriate in view of domestic conditions – *i.e.* low inflation and moderate growth.

Figure 7. **Interest rates and the exchange rate**



1. DM per BF 100.
2. On 2 August 1993, the upper limit was raised to DM 5.63 and the lower limit was decreased to DM 4.17.
3. Three-month Euro-BF.
4. Long-term government bonds (10-year).

Source: National Bank of Belgium and OECD Secretariat.

Given the unsettled conditions in world financial markets and the positive outlook for inflation, monetary and financial conditions in the euro area, on balance, seem unlikely to change much in 1999-2000. Nonetheless, as the economic expansion matures, both short and long-term interest rates may edge up towards the end of the projection period – a development which would be broadly in line with Belgium's domestic requirements. More generally, owing to the very high degree of synchronisation of Belgian economic cycles with those of its neighbouring countries, a monetary policy set at the EMU-wide level should pose no major problems for Belgium.[18]

Budgetary policy

Progress in fiscal consolidation

The medium-term goal of meeting the Maastricht criteria in the fiscal area was formalised by the 1992 Convergence Plan, which was introduced when the current government coalition came to power in an environment of deteriorating macroeconomic conditions and a widening budget deficit. The Plan set annual targets for the progressive reduction of the general government deficit to 3 per cent of GDP in 1996 – compared with a deficit of 6.9 per cent in 1992. To ensure that under reasonable macroeconomic conditions the 1996 target would be reached, the Plan was based on three major features or "norms": the social security system must be in equilibrium;[19] fiscal receipts must display "unit elasticity", *i.e.* they must increase in line with nominal GDP; and primary expenditure of the federal authorities must not increase in real terms. The Plan also set annual targets for all subsets of the general government, that is the Federal government and the social security system (the so-called "E*ntité* I"); and the Communities, Regions and local governments (the so-called "E*ntité* II"). To enhance implementation, the Federal government and the Communities and Regions agreed in 1994 to do their utmost to reach their respective targets. In the 1996 A*ccord de coopération*, Communities and Regions agreed to respect the targets proposed by the C*onseil supérieur des finances*, which are that the E*ntité* II budget should be balanced in 1999.

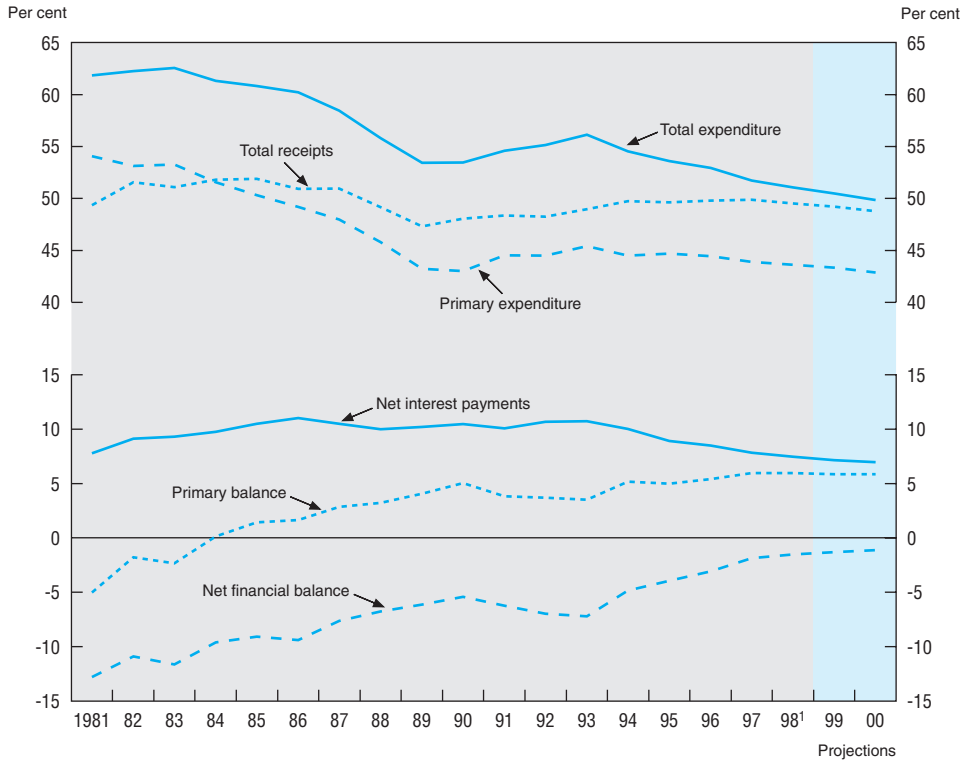
The various norms have broadly been respected and the budget deficit declined to 3.2 per cent of GDP in 1996, *i.e.* close to the 3 per cent target set initially by the Convergence Plan. In the meantime, it had been decided that the year when candidates for Monetary Union were supposed to have brought their deficits down to a maximum of 3 per cent would be 1997. But macroeconomic conditions, on balance, have been less good than expected since 1992, and to offset their negative impact on the budget and keep the deficit reduction on track, the government had to introduce repeated packages of corrective

measures, with a total budgetary saving estimated by the Belgian authorities at a little over 6 per cent of GDP.[20] In late 1996, the Belgian Government presented a new convergence programme covering the period 1997-2000. The new plan stresses the need to keep the primary surplus at around 6 per cent of GDP (on the basis of the Belgian definition) in the medium term.[21] In the context of this new plan an additional package of corrective measures representing over 1 per cent of GDP was included in the 1997 budget,[22] bringing the total corrective effort since 1992 to some 7 per cent of GDP. The lasting impact of this effort on the budget, however, will be significantly smaller than this, since an important proportion of the corrective measures have been of a one-off or non-recurrent nature. This was especially the case in 1996 when over a third of the savings were estimated to stem from one-off measures. In 1997, macroeconomic conditions finally improved markedly, and the general government deficit fell to 1.9 per cent of GDP, representing an improvement of 5 percentage points compared with 1992. At the same time, the primary surplus rose to 5.8 per cent of GDP, above the target set in the 1997 budget and one of the highest in the OECD area.[23]

Reflecting the decline in public debt as a percentage of GDP and the marked decline in interest rates since the early 1990s, which was on very substantial public debt, broadly one-half of the improvement in the budget deficit over this period has been accounted for by a reduction in net interest payments as a per cent of GDP, the other half being accounted for by an increase in the primary surplus (Figure 8). In addition to lower interest rates and a lower debt ratio, the reduction in net interest payments has also reflected a more active debt-management policy – especially in 1993-94 when half of the long-term debt denominated in Belgian francs was refinanced at interest rates which, on average, were 2 percentage points lower. The increase in the primary surplus, in turn, has been mostly accounted for by an increase in receipts and, to a much smaller extent, by a reduction in expenditure (Table 4).[24, 25]

While the decline in interest rates has greatly facilitated fiscal consolidation, the corrective effort over the past six years has nonetheless been considerable. It has included: new direct and indirect taxes and, to a lesser extent, personal social security contributions; cuts in primary expenditure; various other savings, such as reduced interest payments as a result of a more efficient debt management, and additional revenues reflecting, among other measures, improved tax collection and a clamp-down on fiscal fraud. The importance of the corrective effort is confirmed by an analysis in cyclically-adjusted terms – *i.e.* allowing for the impact of macroeconomic activity on the budget – which shows a decline of the overall deficit from over 7 per cent of potential GDP in 1992 – when the economy had a positive output gap estimated by the OECD Secretariat at ½ per cent – to less than 1 per cent in 1997 – when the (negative) output gap was around 2 per cent. This analysis also shows a rise of the primary structural balance from 2¾ per cent in 1992 to 6¾ per cent in 1997. Over this period, fiscal consolidation has been

Figure 8. **General-Government public finances**
Per cent of GDP



1. Estimates.
Source: OECD Secretariat.

less pronounced in the European Union on average: in 1997, the average budget deficit of EU countries was 2½ per cent and 1¾ per cent on a cyclically-adjusted basis – compared with a deficit of 5¼ per cent on both bases in 1992 (Figure 9, Panels A and B). In sum, from a very poor starting point in the early 1990s, Belgium's performance in fiscal consolidation has been rather impressive, and one of the best in the OECD area, especially on a cyclically-adjusted basis.

Looking at fiscal consolidation in terms of the various subsets of the general government, it is clear that the brunt of the corrective effort has fallen on the Federal government and the social security system (*Entité* I). Their combined primary surplus has improved by some 1½ per cent of GDP over the 1992-97 period, with the Federal government accounting for about two-thirds of it,

Table 4. **General government budget**

Per cent of GDP

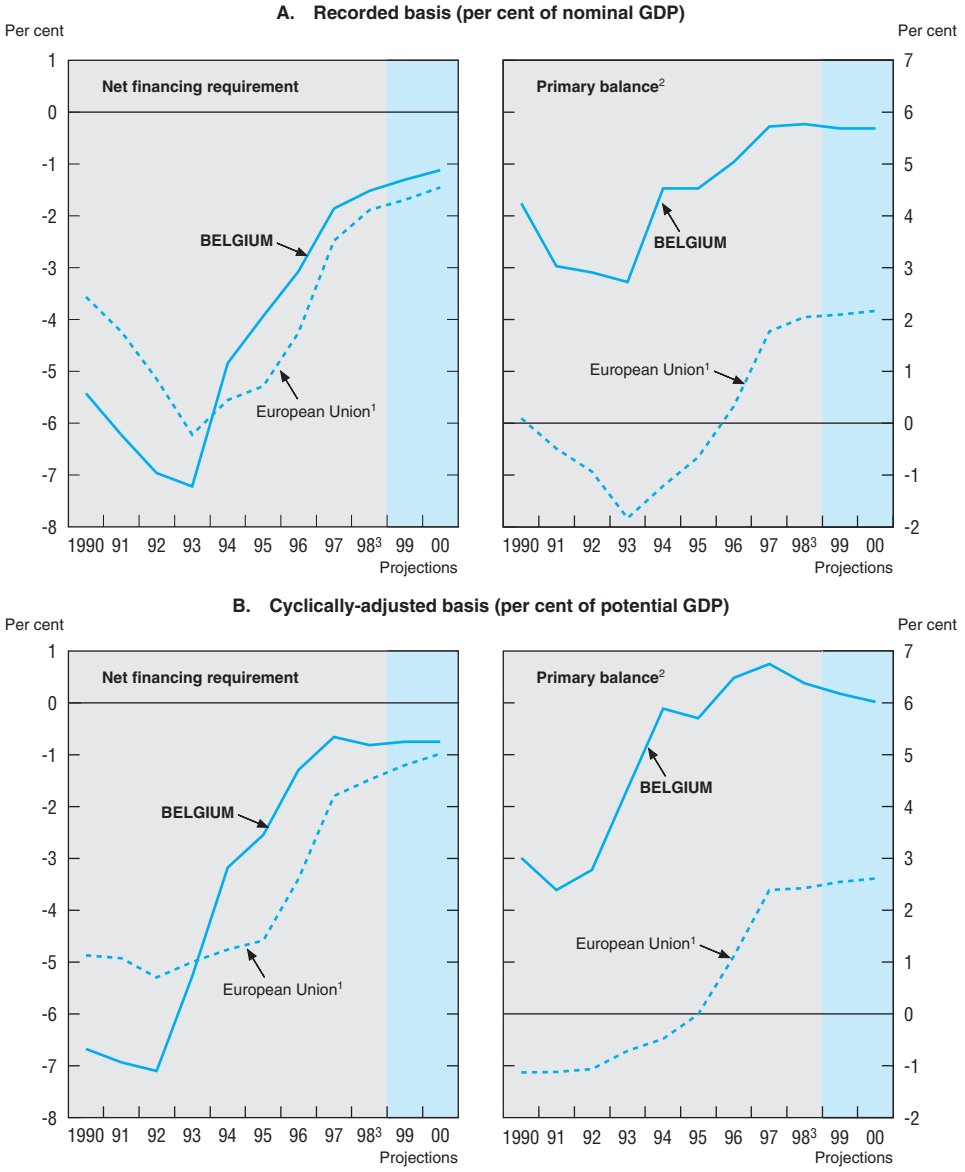
	1990	1992	1995	1996	1997	1998[1]	1999[2]	2000[2]
Primary receipts	47.3	47.4	49.2	49.4	49.6	49.3	49.1	48.6
Of which:								
Direct taxes	16.8	16.3	18.0	18.0	18.4	18.3	18.2	18.0
Indirect taxes	12.2	12.1	12.3	12.7	12.8	12.6	12.7	12.7
Social security contributions	15.0	15.8	15.4	15.1	15.0	14.9	14.7	14.5
Primary expenditure	43.0	44.5	44.6	44.4	43.9	43.6	43.4	42.9
Of which:								
Government consumption	14.0	14.2	14.6	14.5	14.4	14.2	14.1	14.0
Subsidies	2.8	2.7	2.4	2.4	2.0	2.0	2.0	2.0
Social security outlays	20.5	21.6	21.5	21.6	21.2	21.0	20.8	20.6
Primary balance	4.2	2.9	4.5	5.0	5.7	5.8	5.7	5.7
Interest payments (net)	9.7	9.9	8.5	8.1	7.6	7.3	7.0	6.8
Net financial balance	–5.4	–7.0	–3.9	–3.1	–1.9	–1.5	–1.3	–1.1
Memorandum items:								
Cyclically adjusted:[3]								
Primary balance	3.0	2.8	5.7	6.5	6.7	6.4	6.2	6.0
Net financial balance	–6.7	–7.1	–2.5	–1.3	–0.7	–0.8	–0.7	–0.7
Debt[4]	125.7	129.0	130.8	126.7	121.8	117.3	114.5	111.6

1. Provisional figures.
2. Projections.
3. As a percentage of potential GDP.
4. Maastricht definition.

Source: OECD, *National Accounts* and OECD Secretariat.

and the social security system accounting for the remainder (Table 5). Over the same period, the primary surplus of the E*ntité* II (Communities, Regions and local governments) has increased by only ¾ of a percentage of GDP (de Callatay, 1998). This distribution of the adjustment – which has followed the suggestions of the C*onseil supérieur des finances* – would seem reasonable since it is broadly proportional to the budget of each E*ntité*, *i.e.* their respective shares in total primary expenditure. However, the improvement recorded by the E*ntité* II has been the result of an increase in fiscal resources (personal taxes and VAT taxes) allocated to it in line with the L*oi spéciale de financement*. The growth of the Regions' and Communities' primary expenditure slowed appreciably in 1996 and 1997, but between 1992 and 1997 their primary expenditure rose by 1 per cent of GDP. The improvement in the primary surplus of the public authorities as a whole is thus almost entirely attributable to the E*ntité* I.

Figure 9. **Public finances in Belgium and the European Union**



1. Excluding Belgium and Luxembourg.
2. Excluding net interest payments.
3. Estimates.

Source: OECD, *National Accounts* and OECD Secretariat.

Table 5. **Fiscal consolidation since 1992**[1]

Per cent of GDP

	1992	1997	Change 1992-97
Entité I			
(Federal government and social security system)			
Total receipts	33.1	33.5	0.5
Primary expenditure	29.3	28.4	–0.9
Primary balance	3.8	5.1	1.4
Entité II			
(Communities, Regions and local governments)			
Total receipts	12.8	14.6	1.7
Primary expenditure	12.8	13.8	1.0
Primary balance	0.0	0.8	0.7

1. These figures are on a budgetary basis.
Source: Ministry of Finance.

Despite these helpful budgetary developments, the weak point of Belgian public finances has remained the very high public debt as a percentage of GDP. But in this area, too, progress has begun. The authorities have succeeded in setting the debt/GDP ratio on a firmly declining trend – from 135 per cent in 1993 to 122 per cent in 1997. This declining trend has reflected the low level of the government deficit, the narrowing of the differential between the implicit interest rate on the public debt and the GDP growth rate, and the proceeds of privatisation[26] as well as asset management operations which were especially large and reached a total of nearly BF 370 billion in 1996 and 1997. The bulk of this sum (BF 222 billion) represented capital gains recorded by the central bank on previous gold sales which the government was entitled to by law.[27] A further BF 75 billion reduction in government debt resulted from government related agencies investing their excess liquidity in government bonds rather than in bank deposits. All considered, privatisation and asset management operations may have accounted for more than half the reduction in the debt/GDP ratio over the 1993-97 period.

The 1998 budget

The 1998 budget, presented in October 1997, rested, as in previous years, on a cautious macroeconomic scenario from the point of view of both expected growth (2.5 per cent) and the level of short-term interest rates. Because of this and owing to the size of the corrective effort carried out in the past few years and the better-than-expected results for 1997, only a few additional measures were

required to offset higher expected spending in a few areas, such as health care and justice. The new measures – entailing both expenditure cuts and additional receipts – totalled BF 17 billion (or 0.2 per cent of GDP) and included: the extension of levies imposed in 1997 on pharmaceutical companies and electricity producers; and one-off initiatives such as the sale of a license for a third mobile phone operator. All considered, with the fiscal decisions embodied in the 1998 budget, the authorities expect Belgium to be one year ahead in the deficit reduction path set out in the new Convergence Plan and nearly two years ahead with respect to the reduction in the debt/GDP ratio.

With an estimated GDP growth rate somewhat higher than that assumed in the 1998 budget and significantly lower short-term interest rates – 3.6 per cent compared with 4.3 per cent in the budget – the OECD Secretariat estimates the general government deficit to have declined to 1.5 per cent of GDP in 1998, with the primary surplus remaining at around 5¾ per cent of GDP (or at around 6 per cent of GDP on the Belgian definition). On a cyclically-adjusted basis, however, both balances may have deteriorated slightly, due to the end of some one-off corrective measures. The debt-to-GDP ratio may have fallen from 122 per cent in 1997 to 117 per cent – with 80 per cent of the decline attributable to a structural improvement in public finances and 20 per cent to capital gains of some BF 90 billion on a new sale of gold by the National Bank of Belgium (9.6 million ounces) credited to the Treasury and used, as in 1997, to retire foreign currency denominated public debt.

The period 1999-2000

The 1999 budget, which was presented in October 1998, included no new major initiatives. Allowing for multi-annual measures announced in previous budgets, and in view of sustained economic growth, persistently low interest rates and the reduction in public debt, the OECD Secretariat expects the general government deficit to continue to decline – to only a little over 1 per cent of GDP in 2000. However, the primary surplus is projected to remain at around 5¾ per cent of GDP (or about 6 per cent of GDP on the Belgian definition) and, on a cyclically-adjusted basis, this balance may deteriorate slightly, reflecting the end of one-off corrective measures introduced in 1998. The debt-to-GDP ratio is expected to remain on a firmly declining trend, falling to less than 112 per cent of GDP in 2000.

Fiscal consolidation beyond 2000

A rapid reduction in the debt-to-GDP ratio is one of Belgium's policy priorities. To this end, the authorities have committed themselves to keeping the primary surplus (on the Belgian definition) at around 6 per cent of GDP in the medium term. As noted in the 1997 *Survey* (OECD, 1997, pp 46-47), given the high

level of public debt and interest payments as a percentage of GDP, under reasonable assumptions concerning growth and interest rates, a large and broadly stable primary surplus will result in a positive interaction in the progressive reduction in the debt and interest payments as a percentage of GDP – the reverse of the so-called "snowball effect" – which will accelerate the reduction of the overall deficit. The Convergence Plan 1997-2000 introduced by the authorities in late 1996 as a follow-up to the Convergence Plan 1992-1996, provides a concrete medium-term framework for a further sizeable reduction of the deficit and debt levels. It shows that, under the macroeconomic assumptions used in the convergence reports of the European Commission and the European Monetary Institute,[28] the debt-to GDP ratio reaches 60 per cent between 2011 and 2015 – the exact date depending on whether the primary surplus is kept at 6 per cent of GDP until the debt reaches 60 per cent of GDP, or whether it is allowed to decline once the overall balance reaches a given level (equilibrium or a surplus of ½ or 1 per cent of GDP). In the first case, the debt/GDP ratio reaches 60 per cent in 2011: in the last three cases, it reaches that level in 2014, 2013 and 2012, respectively. As these scenarios show, the results are relatively insensitive to the exact interpretation of the commitment to keep the primary surplus at 6 per cent of GDP. The option of allowing it to decline progressively to prevent the overall balance from improving beyond a given level – say one-half of a per cent of GDP – would create valuable room for manoeuvre to meet spending pressure associated with population ageing and accommodate further tax cuts. A change in interest rate assumptions also has only a moderate impact on the results: for instance, an increase in the implicit interest rate on the public debt of 2 percentage points, other things being equal, results in the debt/GDP ratio reaching 60 per cent in 2017 rather than in 2011.

Given the already low level of the overall deficit – an estimated 1½ per cent of GDP in 1998 – and barring exceptional shocks, the commitment to a primary surplus of 6 per cent of GDP would seem more binding for Belgium than the EU's Stability and Growth Pact: this commitment can be expected to bring the overall balance into surplus already in a few years time, while in this area the purpose of the Pact is to prevent deficits from exceeding a level considered as excessive – 3 per cent of GDP – except in the case of economic recession and to bring, over the medium term, the budget balance towards equilibrium or into surplus (see Box 1). But, to avoid having to take corrective (*i.e.* restrictive) fiscal measures during an economic downturn, it would seem appropriate to aim for a somewhat higher surplus when the economy is going through a phase of cyclical overheating.

Another major problem in public finance is the lopsided composition of public expenditure. Especially in the early stage of fiscal consolidation in the 1980s, the burden of adjustment fell disproportionately on public investment. As a result, this item has averaged 1½ per cent of GDP over the past decade – nearly half the EU average and one of the lowest in the OECD area (Figure 10, Panel A).

Box 1. The EU's Stability and Growth Pact

The Pact for Stability and Growth, finalised at the Amsterdam Summit in June 1997, consists of two Council regulations. One clarifies the Maastricht Treaty's provisions for an Excessive Deficit Procedure and the other strengthens the surveillance and co-ordination of economic policies. The Pact also calls on participants in the monetary union to commit themselves to aim at a medium-term budgetary balance or at a surplus.

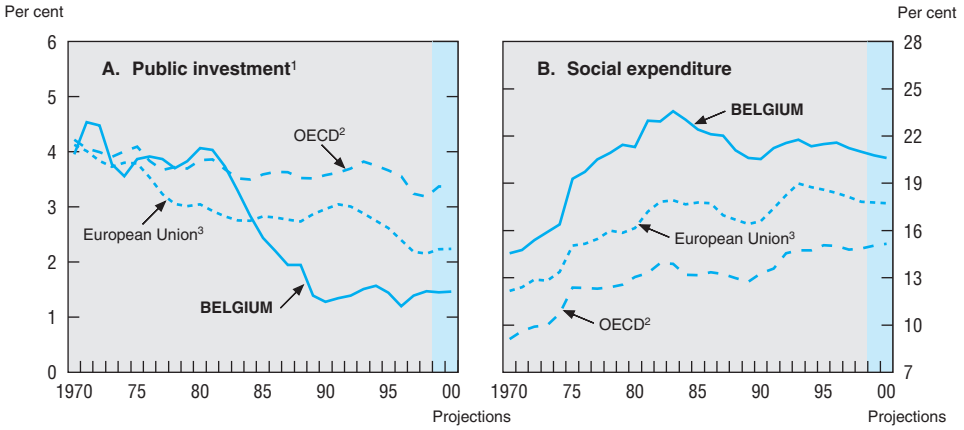
Avoiding excessive government deficits (above 3 per cent of GDP) is considered essential for the success of Economic and Monetary Union. The Maastricht Treaty already included a procedure aimed at discouraging, and reducing when one occurs, excessive deficits. The 3 per cent reference value, however, can be exceeded if: *i*) the origin of the excess lies outside the normal range of situations (exceptionality); *ii*) the excess is limited in time (temporariness); and *iii*) the excess is small enough for the deficit to remain close to the 3 per cent reference value (closeness). These three conditions need to apply simultaneously. The Treaty, however, does specify a precise interpretation of these constraints.

The Stability and Growth Pact gives a more specific interpretation of exceptionality and temporariness. For countries participating in monetary union, the Pact considers a general government deficit above 3 per cent as excessive unless the country is in economic recession. A recession is defined as an annual fall in real output (GDP) of at least 0.75 per cent. Implementation of the Excessive Deficit Procedure depends on the severity of the economic decline. If economic output in a Member State declines by 2 per cent or more, and provided the deficit is temporary, exemption from the Excessive Deficit Procedure is granted. In the event GDP falls by between 0.75 per cent and 2 per cent, exemption can be granted in special circumstances by the Council of Ministers. The country would need to convince the Council that the economic decline was "exceptional" in terms of its abruptness or in relation to past output trends.

Failure to adhere to the Pact could result in the imposition of sanctions. Initially, these would take the form of non-remunerated deposits starting at 0.2 per cent of GDP and a variable component rising in line with the size of the excessive deficit. Such deposits are limited to a maximum of 0.5 per cent of GDP, but would accumulate each year until the excessive deficit is eliminated. Provided the excessive deficit is corrected within two years the deposits are returned to the country. Otherwise, the deposits could ultimately be converted into a fine.

On the other hand, while it is difficult to perform reliable international comparisons of social expenditure, the fact remains that, according to the calculations of the OECD Secretariat, as a percentage of GDP, in Belgium the latter is above the EU and OECD averages (Figure 10, Panel B). To prevent a deterioration of physical infrastructure, and possibly to improve it, it would seem urgent to increase public investment while at the same time curbing social expenditure. There is thus a need to reduce spending on unemployment in all its forms, to step up the

Figure 10. **Selected public-expenditure items**
Per cent of GDP



1. Gross fixed capital formation.
2. Excluding Belgium, Luxembourg, Hungary, Poland and Czech Republic.
3. Excluding Belgium and Luxembourg.

Source: OECD, *National Accounts* and OECD Secretariat.

reform of public sector pensions, and to keep health expenditure under control even if it were decided to drop the "norm" limiting its annual growth in real terms to 1.5 per cent (Chapter IV).

III. Implementing structural reform: a review of progress

Introduction

Within the framework set out by the OECD *Jobs Study*, the 1997 OECD *Economic Survey of Belgium/Luxembourg* provided a set of detailed policy recommendations for Belgium (OECD, 1997, Chapter III and Box 1) to boost non-inflationary economic growth and improve the labour market performance – *i.e.* to increase the employment rate and reduce unemployment in all its forms. These recommendations were derived from a review of structural features of the Belgian economy which determine growth and the incidence of non-cyclical unemployment. The analysis emphasised that in Belgium, as in other European countries, the reasons for unsatisfactory labour market outcomes are multiple, deep-rooted and often interacting.

While the unemployment problem can be traced back to the 1970s, the two oil shocks and the economic, social and political conditions of that time, the persistence of high unemployment and its essentially structural character seem to have been due to a different set of factors, some of them peculiar to Belgium. The most important ones may have been the institutional rigidities and features of the labour market, social protection, and the educational system (broadly defined), which have resulted in a rather inflexible wage structure, heavy non-wage labour costs, insufficient incentives to work, and a lack of appropriate skills and qualifications, especially at the lower end of the market.

The 1997 *Survey* noted that over the previous few years the authorities had taken a large number of measures, but that not all of them had followed a sound and coherent blueprint. For instance, the reduction (albeit on a temporary and selective basis) of the age limit for early retirement, and, above all, initiatives encouraging the unemployed – especially those over 50 – to leave the labour force had been inconsistent with the goal of increasing labour force participation and the employment rate. Moreover, the 1996 law on employment and competitiveness had introduced new rigidities and increased the already considerable degree of government involvement in the labour market. The *Survey* argued that

the complexity and long-lasting character of the unemployment and non-employment problems pointed to the need for a wide-ranging, multi-year strategy, integrating structural reforms in the labour market and the social security system with initiatives to make the whole economy more dynamic and responsive to change. Such a "package deal" was also desirable to exploit synergies resulting from the mutually reinforcing character of most of the required measures and to minimise the unavoidable cost of adjustment. Among the many specific suggestions, the *Survey* stressed the need to:

- make the wage formation system more flexible, especially at the microeconomic level, with a wider wage structure corresponding to differentials in skill and local conditions;
- reform the unemployment insurance scheme and, more generally, the welfare system to increase incentives to return to the active labour market for unemployed and other inactive persons in unemployment-related programmes;
- strengthen and broaden active labour market policies to provide persons in welfare programmes with the necessary guidance and qualifications – although for some persons in early retirement and for some older unemployed the only realistic alternative to inactivity could be work in special social-job programmes.

The *Survey* also pointed out that, while over the preceding years fiscal consolidation and, more generally, joining EMU from its inception had appropriately been the overriding priority of the Belgian authorities, it now seemed possible to shift progressively the focus of attention to the remaining major problem, namely non-employment. The OECD strategy was to be seen as a comprehensive set of first-best measures – a sort of blueprint for the future which would take time to implement fully. During the transition period, second-best measures would no doubt be adopted.[29] It was also emphasised that the strategy should pay due attention to political and social goals, as well as to country-specific factors – in the case of Belgium, the tradition of social consensus and equitable income distribution. Hence, calls for more wage flexibility, less generous social benefits, and a relaxation of employment protection were combined with suggestions to introduce in-work benefits, step up active labour market policies, and consider special social-job programmes for elderly persons on welfare.

The need for a bold and coherent programme of structural reform aimed at improving the functioning of the labour and product markets has been heightened by recent and prospective developments: due to robust growth, the economy is approaching its estimated potential while unemployment – broadly defined – is still high (Chapter I); and the introduction of EMU, combined with the necessity for Belgium to rapidly reduce the debt-to-GDP ratio (Chapter II), means

that fiscal and, above all, monetary policy will largely be unavailable as instruments of economic policy, so that the burden of adjustment to changing economic conditions will primarily fall on the adaptability of the economy, and particularly the labour market. Hence, more than ever, structural reform is the key to higher non-inflationary growth, a sustainable reduction in unemployment, and an economy which can better adjust and adapt to EMU and global requirements.

Progress in structural reform[30]

Since the 1997 *Survey*, the Belgian authorities have pursued their efforts to improve labour market outcomes, operating within the Belgian "mind-frame" which they characterise as emphasising social consensus and the need to prevent the emergence of a class of "working poor" and social exclusion. In this connection, Belgium's poverty rate of 5.5 per cent is amongst the lowest in the OECD (UNDP, 1998). The Belgian authorities acknowledge that technological progress, globalisation, and EMU will require structural reforms in the labour, goods and services and capital markets. More recently, the government has focused its efforts in this area on the preparation of the Belgian National Action Plan for Employment (*Plan d'action belge pour l'emploi*) in connection with the European Employment guidelines. At the special Luxembourg Jobs Summit in November 1997, all EU members agreed to the introduction of 19 employment guidelines with a view to moving towards a more effective and focused European employment strategy. These guidelines are centred around four main themes (or "pillars"): improving employability, developing entrepreneurship, encouraging adaptability of businesses and their employees, and strengthening the policies for equal opportunities. The Belgian government has stated its intention to re-centre its strategy on preventing unemployment, as well as on training and active measures aimed at the reintegration of persons into the active labour force. To prepare the Plan, the government had extensive contacts (*concertation*) with Regions and Communities as well as with the social partners. It was, however, impossible to reach a global agreement with the social partners, and the Plan was finalised by the Belgian authorities. Most of the concrete measures included in the Plan are mentioned below, under the specific sub-headings used in the 1997 *Survey* (Box 2).

Increasing wage and labour cost flexibility

The authorities have made no attempt to revise directly arrangements concerning the administrative extension of sectoral wage agreements which, even when requested by both bargaining partners, force individual employers to set wages that may be at odds with productivity at the firm level, make it more

Box 2. Implementing the OECD Jobs Strategy – an overview of progress

Since the previous *Survey*, a number of policy measures in the spirit of the OECD Jobs Strategy have been either implemented or prepared. However, progress has been slow and uneven: a few important measures have not been considered.

Job strategy proposal	Action taken	OECD assessment/ recommendations
1. Increase wage and labour cost flexibility		
– Encourage *greater wage differentiation* with respect to skills, sectors and firms.		
– Relax arrangements concerning the administrative extension of *sectoral wage agreements and the statutory minimum wage*.		
– Increase the use of "opening clauses", *sectoral wage scales at the statutory minimum wage level, and exceptions to the minimum wage for the various target groups*.	On all these specific points there has been little tangible progress. However, within the framework of the new law on employment and competitiveness, measures have recently been taken to introduce some flexibility, thereby potentially increasing the scope for wage differentiation.	The authorities should consider ways to progress with respect to these specific points, the ultimate goal remaining a phasing-out of the new law on employment and competitiveness and government involvement in the wage determination process in general.
– Introduce *in-work benefits* – such as a means-tested earned income tax credit – to preserve equity or social goals.		

(*continued on next page*)

(*continued*)

Job strategy proposal	Action taken	OECD assessment/ recommendations
– Continue to reduce *non-wage labour costs*, with new cuts targeted at the lower end of the labour market or at groups of persons with poor employment prospects (Maribel cuts targeted at the exposed sectors of the economy should be re-targeted at the lower end of the labour market).	Efforts in this direction have continued and, in the framework of the EU Action Plan for Employment, the government has decided on further cuts over the next six years, representing a total reduction in the average labour cost of 3.4 per cent. A large proportion of the existing and future cuts in employers' social security contributions will be consolidated in a unique scheme which will imply a linear, across-the-board reduction in social security contributions and the strengthening of the measures targeted at the lower end of the labour market (the Maribel measures have been reformed).	Fully implement these measures (as announced in the EU Action Plan for Employment).
2. Reform employment and related benefit systems		
– Both the duration and level of *benefit entitlements* should be reduced to increase incentives to work.		
– *Job-search* controls and benefit *sanctions* need tightening, which requires better co-ordination between the federal body (ONEM) administering benefits and the regional placement services (VDAB, FOREM, and ORBEM).	No major initiatives have been taken since 1996 to reduce the duration and level of unemployment benefits or to tighten job-search controls and the application of benefit sanctions.	Explore ways to move forwards on all these fronts.

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(*continued*)

Job strategy proposal	Action taken	OECD assessment/ recommendations
3. Increase working-time flexibility		
– *Special unemployment-related programmes* – such as "*pré-pensions*" and "*chômeurs âgés*" – should be tightened and progressively phased out. The age limit for early retirement should be raised.	The age limit for early retirement ("*pré-pension*") has been raised, and now stands at 58 (with only a few exceptions).	Press for further increases in the age limit for early retirement.
– *Programmes which favour work sharing* – such as the part-time pre-retirement programme – can be accepted as a second best and for a transition period only, provided they are voluntary, do not increase labour costs, and support employment (in persons).	Measures have been taken to improve the social security position of part-time workers, and the Action Plan for Employment extends the right to take advantage of the special leave programme ("*interruption de carrière*"). Working time has been "annualised", so that seasonal fluctuations will no longer lead to overtime and additional costs for firms.	Continue and step up these efforts
4. Reform employment security provisions		
– *Employment protection* – which in Belgium is largely decided by the social partners – should be relaxed.	Employment protection has not been significantly relaxed, although restrictions on temporary work (*travail intérimaire*) have been reduced and fixed-term contracts made renewable.	Explore ways to ease further employment protection; for example, by allowing fixed-term contracts to be renewed on an unlimited basis.
– *Notice periods* should be shortened for white-collar workers.	No significant changes.	Shorten notice periods for white-collar workers.

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(*continued*)

Job strategy proposal	Action taken	OECD assessment/ recommendations
5. Expand and enhance active labour market policies		
– The *"plan d'accompagnement" and other programmes in charge of active policies* should be strengthened and broadened – for instance to include persons aged over 46 – to provide the necessary skills and guidance to persons affected by the suggested tightening of programmes for early retirement and older unemployed.	The Action Plan for Employment indicates that the *"plan d'accompagnement"* is to be strengthened to provide guidance to young unemployed – after six months of unemployment – and to long-term unemployed – after 12 months. A temporary cut in social security contributions will be offered to firms hiring these unemployed. Other measures have been taken: occupational transition programme, strengthening of the part-time work programme provided by local employment agencies, and Smet-jobs.	Broaden the *"plan d'accompagnement"* to include persons aged over 46. To increase the effectiveness of this plan, contact with job seekers should be more frequent and ways should be found to reduce problems arising from the distribution of responsibilities among the federal government, Communities, and Regions.
– Pursue recent efforts to *evaluate active labour market policies*.	Various official bodies, notably the Ministry of Employment and Labour, have continued their efforts to evaluate active labour market policies.	Continue these efforts.

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(*continued*)

Job strategy proposal	Action taken	OECD assessment/ recommendations
6. Improve labour force skills and competences		
– Reduce *failure at the secondary level of education*. – Monitor the *effectiveness of recent reforms* of the elementary and secondary education levels. – Initiate *further reforms* to complement ongoing efforts to simplify and clarify teaching goals; diversify curricula and teaching methods; and strengthen remedial education as an alternative to repeating years.	The Flemish government has recently started a major reform of secondary education, aiming at streamlining study options, enhancing guidance and counselling pupils, and better preparing teachers and school managers. At the primary level of education, the Flemish Community is especially interested in risk groups: it is improving the school admission policy for immigrants, and allocating additional staff to improve the educational opportunities of these pupils. The French Community has undertaken a reform of technical and vocational secondary education and also of general secondary education, in the first case defining skill profiles and developing teaching tools and, in the second, revising all the curricula in order to ensure uniform quality. In primary education, a plan targeting schools at risk has been implemented, with more teachers and more material assistance, while the children of illegal residents are able to attend school in the normal way.	Implement these reforms; monitor and assess their results.

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Job strategy proposal	Action taken	OECD assessment/ recommendations
– Further expand *apprenticeship training* through reinforced education-industry partnerships.	The system of industrial apprenticeship (offering part-time training and part-time work) has been extended and made less expensive for employers. As a counterpart to further cuts in employers social security contributions (mentioned above), the social partners have committed themselves to increasing expenditure on training. The regions have also taken measures relating to occupational training.	Fully implement these commitments, and consider whether the education-industry partnership concerning apprenticeship training could be further reinforced.
7. Enhance the creation and diffusion of technological know-how		
– Improve the *financial infrastructure* which supports the commercialisation of private research, with a view to enhancing R&D and innovation.	The financial infrastructure has not changed substantially, but Flanders has introduced a "Guarantee Fund for Risk Investment" and Wallonia is expected to introduce a loan guarantee scheme to promote the commercialisation of R&D results.	Consider ways to further strengthen the financial infrastructure supporting the commercialisation of private research.
– Nurture an *entrepreneurial climate* by, among other things, cutting red tape at all levels of government, which seems to be especially cumbersome for SMEs.	The government has taken or is preparing a number of measures in this area, including: a legal package in favour of SMEs, and an Agency for Administrative Simplification to streamline existing legislation and assess the likely impact on business of any new proposed legislation.	Fully implement the announced measures and assess recent and future measures in this area.

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Job strategy proposal	Action taken	OECD assessment/ recommendations
8. Enhance product market competition		
– *Provide the recently established Competition Council and other relevant authorities with sufficient staff* to perform their statutory role in the implementation of the 1993 Competition Act.	The Competition Council has been given additional staff and resources, and the Competition Act has been revised to allow more emphasis to be put on protection against the abuse of economic power rather than on concentration.	Monitor and assess the results of these measures.
– *Entry conditions in the distribution sector should be liberalised*, diminishing the power currently exercised by incumbent shop-owners to limit new entry of outlets.	No significant changes.	Explore ways to move forwards on this front.
– *Rules on shop-opening hours should be relaxed*, to enhance both consumer choice and competition.	"Night shops" have been legalised but legislation in this area remains rather restrictive.	Rules on shop-opening hours should be relaxed further.
– *Reassess the regulatory framework* with a view to streamlining and liberalising existing rules.	As noted in point No. 7, an Agency for Administrative Simplification is to be introduced to streamline existing legislation and assess the likely impact on business of any new proposed legislation.	
– *Privatisation* of companies in which public ownership remains important – *i.e.* in telecommunications and air transport – should be pursued further.	Some progress has been recorded, but in the electricity sector there continue to be serious competition problems.	Continue with privatisation efforts in general, and more particularly explore ways to reduce Electrabel's near monopoly position in both the production and distribution of electricity.

difficult for new firms to break into established markets, and hence may prove detrimental to employment. "Opening clauses" allowing firms, under certain circumstances, to waive given provisions in collective contracts at the branch

level, have reportedly been introduced in some manufacturing sectors but their effective use appears to be very marginal. Similarly, there seems to have been no significant relaxation of economy-wide and sector-wide minimum statutory wages[31] and effective wage floors stemming from the welfare system which may price a large number of low-skilled and young workers out of the market – although some new entrants who are less than 30 years old are allowed to start working at 90 per cent of the "normal" wage,[32] and minimum wages are differentiated by age, *i.e.* are lower for those under 21 years old.

Within the framework of the 1996 law on employment and competitiveness – which aims to limit, on an *ex ante* basis, the maximum increase in labour cost (compensation per employee) in the private sector to the expected weighted average increase in the three reference countries, *i.e.* Germany, France and the Netherlands[33] – three measures have been or are about to be introduced to increase wage differentiation, at least by allowing higher wages, thereby partly responding to the criticism that the new law increased rigidities at the micro level. The measures are:

- a profit-sharing scheme giving firms that have increased employment the right to grant an additional remuneration to their employees, over and above the economy-wide upper limit set for ordinary wage increases;
- a favourable tax and quasi-tax regime for stock options;
- and a favourable tax and quasi-tax regime in respect of the discount granted to employees when there is a new equity issue.

Moreover, in the Action Plan for Employment, the government has pledged to introduce a system of participation of employees in equity capital, and an equivalent system (savings account system) for small businesses.

The suggestion of lowering wages for low-skilled workers to price them back into the market is still regarded as largely unrealistic in Belgium. Hence, there has been little interest in exploring the possibility of combining this approach with the use of employment-conditional or in-work benefits – such as an earned income tax credit – to preserve equity or social goals. Admittedly, as noted in the 1997 *Survey*, for members of specific target groups – such as youngsters with no job experience and lacking the required work attitude or older long-term unemployed – it is probably unrealistic to count on market forces, even combined with active measures, to return most of them to the active labour force. It might be better to encourage some of them to be active, at least on a temporary basis, in special subsidised jobs, performing work of social interest combined, when appropriate, with some training. In this respect, the Belgian authorities have introduced a new programme which uses part of the normal unemployment benefit to subsidise jobs with low productivity (the so-called Smet-jobs). Another form of encouragement is the occupational transition programme in the

non-market sector. In addition, the authorities have strengthened the programme allowing local employment agencies (*Agences locales pour l'emploi*) to offer part-time jobs with limited salaries to long-term unemployed (who retain their unemployment benefits). Other measures have been taken by the Regions and Communities.

The system of automatic wage indexation has not been modified. However, the authorities consider that its effects have been significantly curbed by the "global plan" of 1993, which removed tobacco, alcohol, petrol and diesel fuel from the price index used to calculate wages and social benefits. Moreover, in their view, the relevance of the indexation system has been further reduced by the law on employment and competitiveness which sets maximum rates of growth for labour costs in nominal terms. However, under the law on employment and competitiveness, indexation and the wage drift have "priority", at least in the short term – *i.e.* they are fully permitted even if they lead to wage increases above the limit set by the law. It remains to be seen how the system would work if, as a result of indexation and the wage drift, labour costs were to increase more than the maximum allowed by the law for a period of several years.[34]

A progressive reduction in non-wage labour costs has been the centrepiece of the government's labour market policy in recent years. These efforts, predating the National Action Plan, have been pursued, with cuts in employers' social security contributions targeted at specific labour market groups – *i.e.* low-wage workers, youths, and long-term unemployed – and partially compensated by increases in various special contributions on taxable income and indirect taxes (the so-called "alternative financing" of the social security system). Cuts in employers' contributions have reached some BF 60 billion a year, representing a reduction in the average labour cost of about 2 per cent. The Action Plan for Employment includes additional cuts in employers' contributions of BF 18 billion a year, representing a total reduction in the average labour cost of 3.4 per cent over six years (from 1999 to 2004). As a counterpart, the social partners are expected to take appropriate – but as yet not fully specified – measures to improve training and enhance employment. These new cuts will be consolidated with existing ones and extended to all wage-earners.[35] The new consolidated scheme will include: a general per worker cut in employers' social security contributions for all workers in all sectors; and an additional cut for low-wage workers. The temporary cuts targeted at specific groups – such as young unemployed and long-term unemployed – remain in force.

Reforming unemployment and related benefit systems

No major initiatives have been taken since 1996 to reduce the duration and level of unemployment benefits in order to increase incentives to work and enhance the employment of low-skilled workers. As noted in the 1997 *Survey*, the

long duration of unemployment benefits remains a hallmark of the Belgian system[36] and, although benefits are frozen in real terms, the average measure of benefit entitlements is among the highest in the OECD area. Although initial replacement rates are not particularly high compared with other European countries, overall average rates are among the highest in the OECD area, according to standard calculations by the OECD Secretariat (OECD 1997, Belgium, p. 70 and Figure 17). However, these calculations by the OECD Secretariat are for unemployed with a rather long professional career (22 years). Calculations by the Belgian authorities for unemployed with a shorter career show lower levels of replacement rates. Earlier in the decade, the authorities focused their corrective efforts in this area on the use of article 80 which stipulates that, under certain conditions, beneficiaries can be suspended from receiving unemployment benefits if their unemployment spell is "abnormally long". The definition of an "abnormally long" spell was reduced (to 1½ times the average duration of unemployment in the local labour market) and the overall implementation of article 80 was tightened, resulting in the suspension of a significant number of persons from the unemployment scheme. But no further tightening has taken place in the recent period, and suspensions on account of "abnormally long" unemployment have declined. Article 80 still covers only a limited group – unemployed persons aged under 50 who are in a household with a second income earner (*cohabitant*) and have a combined income of more than BF 600 000 – and it allows for several exceptions.

The more effective use of article 80 earlier in the decade also allowed a sort of indirect control on job search with corresponding sanctions. This was an especially valuable aspect given the need for Belgium to strengthen direct surveillance of job search, availability rules, and sanctions through better co-operation and co-ordination between the federal body (ONEM) administering benefits and the regional placement services (VDAB, FOREM and ORBEM) – the long-standing position of the latter being that surveillance of job search is not their priority task.

Increasing working-time flexibility

The age limit for early retirement ("*pré-pension*"), which a few years ago had been reduced to 55 (under certain conditions), has been raised again, and now stands at 58 (with only a few exceptions). Measures have also been taken to improve the social security position of part-time workers, which should enhance both this type of work and part-time pre-retirement. Moreover, the Action Plan for Employment extends the right to take advantage of the special leave programme ("*interruption de carrière*"). These programmes which favour work sharing offer the advantage over other job-related programmes (such as "*pré-pension*" and "*chômeurs âgés*") of keeping their members in contact with the labour market so that, in case

of a sustained economic upturn, these part-time workers could easily resume a full-time status. Hence, although far from ideal, they may be acceptable as a second-best policy and for a transition period – *i.e.* until more appropriate measures can be implemented – provided they are voluntary and do not increase labour costs. Working time has been "annualised", so that seasonal fluctuations will no longer lead to overtime and additional costs for firms, and measures have been taken at Federal and regional level to encourage an overall reduction in working time. Finally, the government has asked the *Conseil supérieur de l'emploi* to consider how to increase the employment impact of measures encouraging work-sharing – such as a reduction in working-time and an increased use of part-time jobs – and to review the experience in this area of France, Germany and the Netherlands.

Reforming employment security provisions

Employment protection – which in Belgium is largely decided by the social partners – has not been significantly relaxed, although restrictions on temporary work (*travail intérimaire*) have been reduced and the possibility of having successive contracts of limited duration has, after an experimental period, been made permanent.[37] As noted in Chapter I, temporary work has grown rapidly over the past couple of years. Notice periods for white-collar workers should be shortened.

Expanding and enhancing active labour market policies

As indicated in the Action Plan for Employment, the *plan d'accompagnement des chômeurs*, which is Belgium's main initiative in the area of active labour market policies, is to be strengthened. An agreement has been reached between the federal government, the regional governments, and the social partners on a comprehensive strategy to provide, through this plan, guidance to young unemployed – after six months of unemployment – and to long-term unemployed – after 12 months – directing them either to schooling, training and on-the-job experience programmes or to a job. A temporary cut in social security contributions will be offered to firms hiring these unemployed. Amongst other measures, mention should be made of the occupational transition programme, local employment agencies and Smet-jobs. While these initiatives are welcome, the effectiveness of the *plan d'accompagnement*, as noted by the Ministry of Employment and Labour (1997) in its valuable effort to monitor and evaluate labour market programmes, may continue to be hindered by the distribution of responsibilities among different levels of government which do not necessarily share the same priorities – the federal government and the social security system financing unemployment benefits and the *plan d'accompagnement*, the Communities being responsible for education and training, and the Regions for job search and employment (*i.e. placement des travailleurs*).

Improving labour force skills and competences

Both the French and Flemish Communities have pursued their efforts to correct the shortcomings of the educational system – broadly defined to include vocational education (apprenticeship programmes). The Flemish government has recently started a major reform of secondary education, aiming at streamlining study options, enhancing guidance and counselling of pupils, and better preparing teachers and school managers. Concerning the latter point, model job descriptions have been worked out for 14 different groups of jobs in secondary education. They can be used by individual schools to prepare job descriptions that fit their own educational and professional needs. Moreover, initial and in-service training programmes will be strengthened. As for the curriculum, by bringing together similar options in different fields of study and eliminating outdated and overlapping options it is hoped to achieve greater transparency in secondary education and allow more efficient guidance and counselling of pupils. With regard to guidance and counselling, the centres for psychological and medical counselling will be substantially reformed, integrating educational and career guidance with medical care – with the staff focusing on children at risk at specific points of transition during compulsory schooling. Finally, to meet some of the weaknesses of vocational education, the authorities intend to make this form of education more flexible, allowing pupils – especially those at risk – to proceed at their own pace and choose among various curricula. Concerning repeaters in secondary education in Flanders, a recent partial study covering the first half of the 1990s has shown a generally declining trend of repeaters as a proportion of total pupils.

The French Community has undertaken a comprehensive reform of its technical and vocational secondary education. The reform is based, first, on a definition, which was formulated jointly with representatives of the business community, of skills profiles reflecting the abilities that are expected of young graduates. Based on these profiles, pedagogical tools and banks of evaluation tests are being gradually constituted to ensure uniform quality. Henceforth, within the French Community, the same skills and the same diplomas as in the mainstream secondary system can be obtained through alternating classroom/workplace programmes, whose cost to employers has been cut in line with the learning model for the self-employed (''*classes moyennes*''). General secondary education is also being extensively reformed, with all curricula being reviewed in order to better ensure a similar level of study in all schools. Based on the new curricula, all programmes are to be reviewed and approved so as to be tailored to the new requirements. Teaching tools are to be pooled for all establishments. Basic education was given priority, with a multi-year reinvestment plan of over BF 4 billion. In co-operation with the Regions, the French Community is equipping all of its secondary schools with cybermedia classrooms as part of a multimedia plan. The

underachievement rate, which had been particularly high in the French Community, reversed its downward trend, thanks in particular to the adoption by the lower secondary system of advancement by degree rather than annual promotion.

At the primary level of education, the Flemish Community is especially interested in risk groups: by enhancing local agreements on non-discrimination, the authorities intend to improve the school admission policy for immigrants. Schools with a large number of immigrants are allocated additional staff to improve the educational opportunities of these pupils. Also, more teachers have been hired and trained to enhance the transition of children at risk from nursery school to primary school. The French Community has instituted an aid programme targeting schools considered at risk, which are getting more educators and teachers, along with material assistance for refurbishment and for setting up libraries. The plan required an annual investment of BF 1 billion. Concerning the sensitive issue of children of illegal residents (the "*sans-papiers*"), the French Community has authorised and is providing the funding for such children to attend on an equal footing with its own citizens.

The national system of industrial apprenticeship (offering part-time training and part-time work) has recently been extended and made less expensive for employers. As noted above, in the framework of the Action Plan for Employment and as a counterpart to further cuts in social security contributions, the social partners have committed themselves to increase expenditure on training, and bring it up to the average level of the three major trading partners – Germany, France and the Netherlands.

Enhancing the creation and diffusion of technological know-how

With a view to improving the financial infrastructure which supports the commercialisation of private research, Flanders introduced in July 1997 a "Guarantee Fund for Risk Investment" to protect the capital of investors in future-oriented growth companies. Wallonia is expected shortly to introduce a loan guarantee scheme, endowed with some $25 million, to promote the commercialisation of R&D results. Also, a network has been installed in the four sectoral centres for research to inform enterprises about patent procedures and existing patents.

Several measures have been taken or are being prepared to cut red tape, nurture an entrepreneurial climate and promote small and medium-sized enterprises (SMEs). They include:

- a legal package in favour of small businesses to relax the access to regulated professions, offer fiscal and financial incentives (also for R&D expenditure), and cut social security contributions for new recruitment;
- an Agency for Administrative Simplification (under the direct supervision of the Prime Minister's office) to assess the likely administrative impact on business of any new proposed legislation, and streamline existing legislation and regulations;

- a "single administrative window" ("*guichet unique*") for SMEs;
- a social security identity card to simplify the administration of the social security system and controls in this area;
- social and fiscal ruling, *i.e. ex ante* agreements between enterprises and the administration on social security and fiscal matters;
- a co-ordinating agency for foreign investors (Federal Agency for Foreign Investment), combined with streamlined procedures for foreign investors, and advice in this area at major diplomatic posts abroad;
- an *ex ante* tax treatment of stock options;
- non-taxable discounts – up to 20 per cent – for employees buying shares in their own company;
- and the introduction of a single identification number so that the information required can be put together in all the administrative data banks in such a way as to reduce the number and content of the forms to be filled in by companies.

Increasing product market competition

Increased competition in product markets is an essential component of the job strategy. This is especially true for Belgium, where reform in this area could enhance the movement of labour away from declining industries and firms and towards the most dynamic segments of the economy, thereby facilitating the easing of many restrictive labour market arrangements. Since April 1993, Belgium has had a strict and comprehensive competition legislation, on an equal footing with European Union law and the national law of most other Member countries. However, a lack of resources – among other things – seems to have prevented the Competition Council (*Conseil de la concurrence*) and the other relevant bodies to perform fully their statutory role in the implementation and supervision of the 1993 Competition Act. The Competition Council has taken relatively few decisions on practices that restrict competition, and has not always given its decisions within a reasonable period of time. Recently, the authorities have taken measures to correct this situation, providing the Competition Council and Competition Service with additional staff and resources. Also, the Competition Act has been revised to allow more emphasis to be put on protection against the abuse of economic power and other restrictive practices rather than on concentrations: to limit the potential number of concentration cases to be examined, the thresholds determining potentially unlawful concentration have been changed and will be applied only to activity in the Belgian market.

Little seems to have been done to liberalise entry conditions in distribution. As regards rules on shop-opening hours, they have been somewhat relaxed: "night shops" have been legalised and Sunday shop-opening has been allowed under certain conditions. But in this area too, the legislation remains rather restrictive. For instance, "night shops" cannot be open during the day.

Concerning privatisation, 25 per cent of the financial company ASLK/CGER has been sold to Fortis and legislation has been introduced to sell the remaining 25 per cent public holding. Legal and regulatory decisions have been taken to allow the public sector airport operator (RVA/RLW) to merge with a private company – Brussels Airport Terminal Company (BATC) – and list the new company (BIAC) on the stock exchange. In the electricity sector there are serious competition problems since Electrabel has a near monopoly position and dominates all aspects of the industry, notably the production of electricity and its distribution. The authorities have recently taken measures transposing the European Directive on liberalisation of the electricity sector. A period of perhaps as much as ten years has been scheduled to allow for a progressive transition and adjustment to competitive conditions.

Assessment and scope for further action

The past year and a half has seen a number of developments which can be expected to improve the functioning of the labour and goods markets. On the whole, however, progress has been slow and rather uneven. This is not entirely surprising: OECD's job strategy was presented as a sort of blueprint for the future to get the discussion started, but which would take time to implement. Also, the approach adopted by the Belgian authorities has fitted into the European framework which, for the time being, does not cover, among other points, the wage determination process and incentives to work, and can thus largely be seen as a subset of the jobs strategy.[38] Within this EU framework, the Belgian authorities have continued to emphasise cuts in employers' social security contributions, appropriately focused on low-wage workers, combined with "active" measures – such as training and guidance. The cuts planned over the next six years are expected to bring employers' social security contributions fully in line with those of Germany, France and the Netherlands, on average – the target of the Belgian authorities. Given the continuing need for fiscal consolidation to reduce the debt-to-GDP ratio, the scope for further cuts seems rather limited, especially when compared with the size of the problem in the labour market.

It is true that recent macroeconomic outcomes in this area have been encouraging. Compensation per employee in the private sector has increased broadly in line with that in the three reference countries – Germany, France and the Netherlands – and the standardised unemployment rate has declined significantly and is below the EU average. But these results should not lead to complacency. The decline in unemployment has largely reflected cyclical developments, even if structural measures – such as cuts in non-wage labour costs – may have increased the labour content of growth; and the standardised unemployment rate, while below the EU average, is well above the OECD average and still high

by historical standards. Moreover, the decline in the claimant-based unemployment rate (national definition) has partly been the result of administrative decisions leading to the withdrawal of many old unemployed from the labour force – and hence from unemployment as commonly defined (see Chapter I). With persons in the early retirement programme and in other unemployment-related programmes, these old unemployed partly account for the low rate of employment in Belgium. As for the modest increase in wage costs, this is not surprising given the law on employment and competitiveness. The 1997 *Survey* noted that the new law "should be much more binding than the previous one" (OECD, 1997, p. 86), *i.e.* the 1989 Law on Competitiveness.[39] But this macroeconomic wage moderation may come at a price – that is, the long-term danger of potentially large harmful effects arising from rigidities introduced by the law at the microeconomic level. Even if the profit-sharing scheme discussed above succeeds in introducing some upside flexibility, downside rigidity will remain unchecked. In addition to a misallocation of resources, this situation will progressively lead to pent-up tensions, with the risk of an excessive correction at a later stage. Hence, the law on employment and competitiveness should be progressively phased out before the slow-building and difficult-to-detect harmful microeconomic effects seriously threaten the working of the economy and undermine potential growth.

To prepare for a phasing out of the law on employment and competitiveness without jeopardising wage moderation at the macroeconomic level, the focus of attention of employment policy should include, in addition to reducing non-wage labour costs, also freeing the wage formation process, especially at the low-wage end of the labour market. Virtually all the specific recommendations in this area presented in the 1997 *Survey* are still relevant. For instance, it was noted that several provisional measures could facilitate the transition to a more flexible wage formation process, allowing downward wage adjustments at the microeconomic level and thereby increasing the employment prospects for young people and other low-skilled workers: the social partners when negotiating collective agreements should lower the minimum sectoral wage to the minimum statutory wage level; greater use could be made of "opening clauses"; and existing exemptions from the statutory minimum wage could be generalised to employers hiring various target groups – such as older persons, youngsters, or long-term unemployed. More generally, easing job protection rules and discontinuing the practice of administrative extension of collective agreements would help in reducing the power of insiders – *i.e.* persons with a job – in the wage determination process and allow for more flexibility.

It was also stressed in the 1997 *Survey* that, to preserve social and equity objectives and avoid creating "working poor" while also strengthening work incentives, these job-creating measures should be accompanied by a reform of social protection. A combination of a wider wage distribution and in-work benefits – such as an earned income tax credit – could help to create employment

opportunities for low-skilled workers and people who have prematurely withdrawn from the labour force, enhance incentives to work, and assure a minimum standard of living. The implications for public finance of a shift of persons from a passive benefit situation to low-paying jobs complemented by in-work benefits are difficult to calculate but, all considered, might well be positive in the medium term; and, anyway, returning these people to the active labour force is a priority, lest they become permanently unemployable because of age, obsolete qualifications, or a general loss of attitude to work. Nonetheless, given that virtually no OECD countries have yet experimented on a large scale with increased wage flexibility combined with in-work benefits, this approach should be followed in a pragmatic and flexible way; and since even earned income tax credits would raise effective marginal tax rates over a range of the income distribution, care should be taken to minimise the risk of "poverty traps" – *i.e.* circumstances in which workers earning low pay have little or no incentives to raise their earnings.

Current efforts to step up active labour market policies should be pursued. With a view to facilitating the return of some of the older persons currently in special unemployment-related programmes to the active labour force, the obligation to participate in the *plan d'accompagnement* should be extended to persons aged over 46; and to enhance the effectiveness of this plan, contact with job seekers should be more frequent and ways should be found to reduce problems arising from the specific Belgian administrative set-up, which requires a distribution of responsibilities among the federal government, Communities, and Regions. Moreover, as many older inactive persons with obsolete skills may be largely "unemployable" – *i.e.* it may be difficult to return them to the active labour force, regardless of their wage level – it might be appropriate to offer to some of them special subsidised jobs, performing work of social interest. For other target groups of persons who would otherwise also be "unemployable", such as youngsters with no job experience and lacking the required work attitude, the authorities have already introduced similar programmes on a rather limited scale. These could be extended progressively, and the jobs offered ought normally to be temporary and combined with training programmes.

As little progress has been recorded in reforming unemployment and related benefit systems, most of the recommendations presented in this area in the 1997 *Survey* remain valid. Efforts should be pursued to curb the duration of earnings-related unemployment benefits, if not through the use of article 80 – revised anew to widen its scope and speed up its application – then through other initiatives: co-operation and co-ordination between the federal body administering benefits (ONEM) and the regional placement services (VDAB, FOREM and ORBEM) have been strengthened with respect to active employment policy and may also be with respect to checks on availability; above all, however, job-search controls and the application of benefit sanctions should be stricter. Although sanctions in Belgium are severe compared with many other OECD

countries, their application, for example in the case of refusal of suitable work, is ineffective for legal as well as institutional reasons. Moreover, the age limit for early retirement should be increased further and, more generally, ways should be explored to tighten or phase out special unemployment-related programmes, such as "*pré-pensions*" and "*chômeurs âgés*" – with the help of active policies and subsidised jobs.

For social and political reasons, increasing wage flexibility and redressing incentives to work has proved a difficult and slow process. It might thus be appropriate to concentrate policy efforts also on improving competition in product markets. For instance, the current licensing procedure for the establishment of shops acts as an excessive barrier to entry, favouring incumbent shop owners without necessarily advancing societal objectives in such areas as urban planning. Hence, entry conditions in the distribution sector should be liberalised; and rules on shop-opening hours should be relaxed further. These are two areas where the scope for improvement seems especially large, and where deregulation holds the promise not only of improving consumer choice and competition, but also of enhancing private sector job creation. This would facilitate the task of making the labour market more flexible which, in turn, is a crucial condition for reaping the full benefits of product market reform. Experience in other Member countries has shown that product market reform is likely to cause shocks to employment – positive in certain sectors and negative in others.[40]

In the electricity sector, EU directives will impose a certain amount of competition in 1999, but more will be required to put this sector on a solid competitive foundation. As a first step, the Committee for the Control of Electricity and Gas should be turned into an effective supervisory agency with a clear mandate to protect the interests of all consumers. The implementation of the decisions taken recently concerning the liberalisation of the electricity sector should have the effect of breaking up Electrabel's dominance in the areas of production and distribution – or, at the very least, making access to the transmission grid open and fair.

IV. The health care system

Introduction

The Belgian health care system has never been considered in depth by the EDRC. In 1992, a study by the Directorate for Education, Employment, Labour and Social Affairs (OECD, 1992) noted that the Belgian health system demonstrated many strengths: it provided comprehensive health insurance cover to the entire population, ensuring high levels of quality and equity; patients were free to choose both their insurer and their service provider; and, in comparison with neighbouring countries, Belgium seemed to achieve a similar performance in terms of health outcomes while spending a slightly lower share of its GDP. However, as in many other OECD countries, it is in the health sector that public spending has risen most rapidly in the 1980s and in the first half of the 1990s. The Belgian government has repeatedly taken measures to restrain the growth of expenditure in this sector, but the cost of health care and its financing has remained a major policy issue, and questions have increasingly been raised about the long-term viability of the system in its present form, especially in view of the expected impact of population ageing and the rapid introduction of new and more expensive medical technology. It may thus be appropriate to review in some detail the Belgian health care system, and consider how it could be reformed to strike a better balance between, on the one hand, the need to slow the rate of increase in expenditure and make the system more efficient and, on the other, the objective of preserving its characteristics of quality and equity, which are strongly supported by a large majority of Belgian society.

This chapter is divided into four sections. After presenting, in the first section, a brief overview of the historical evolution of the Belgian health system, its main features – in terms of institutional aspects and outcomes – are identified in the second section. The third section considers recent reform efforts and developments; and the final section discusses current and prospective problems, some major policy issues, and recommendations for further policy initiatives.

Historical overview

The key features of the Belgian health care system largely reflect decisions which were progressively taken in the period following the Second World War: the Law of 1963 completed the introduction of a compulsory public health insurance combined with a private system of delivery of health care based on independent medical practice, free choice of doctor and hospital, and (reimbursed) fee-for-service payment. The management and administration of health insurance was essentially entrusted to non-government non-profit organisations, *i.e.* mutual sickness funds or *mutualités*, with a large role for *concertation* – for instance between *mutualités* and doctors to determine fees and reimbursements. The role of government was limited to the regulation and partial funding of the system and to the pricing of drugs and the setting of hospital rules and budgets. The Law of 1963 was the result of a broad social consensus on the desirability of providing quality medical care to everybody on an equal basis, and ensuring a high degree of freedom of choice to both patients (in choosing their doctors) and providers (in deciding on the appropriate therapy).

In the hospital sector, the authorities took steps to curb the growth of expenditure as early as the mid-1980s. The continuing rapid increase in health costs, combined with the pressing need for fiscal consolidation, obliged the authorities to undertake a number of reforms – notably in 1992-93 – to curb the growth of public health spending, while preserving as much as possible the underlying principles of equity, freedom of choice, and quality of care. Hence, as discussed below, corrective efforts have focused on eliminating abuse and waste, and raising the efficiency of the system by increasing the cost-consciousness (*responsabilisation*) of all major players and modifying their behaviour. But corrective measures have also included a stringent "norm" limiting the annual growth of health care expenditure in real terms to 1.5 per cent, as well as the introduction of a few significant exceptions to the principle of fee-for-service and selective increases in the co-payment or cost-sharing of patients (*ticket modérateur*).

The process of federalisation of the country – which has taken place over the past two decades or so – has not greatly affected the compulsory health insurance system: being part of the social security system, it has remained the responsibility of the federal government. On the other hand, the responsibility for some other aspects of health care has been shifted to the Communities and Regions. These aspects include: preventive care, health education, and the implementation of various decisions initiated at the federal level – such as initiatives concerning the number and size of hospitals.

Main features of the system

Institutional aspects

Overview

A distinguishing characteristic of the Belgian health care system is that it combines compulsory, comprehensive, and universal public health insurance with patients' free choice and independent medical practice (*médecine libérale*). It rests on the solidarity principle, since, as a rule, contributions are proportional to participants' revenues – with no reference to the health risk – while reimbursements are equal for everyone – except for especially vulnerable social groups.[41] The compulsory health insurance scheme is an integral part of the Belgian social security system. It covers major risks for the whole population and minor risks for nearly 90 per cent of the population.[42] It is administrated by five *mutualités*,[43] a fund for the railways and one public fund, which are entrusted with the role of providing compulsory insurance, reimbursing fee-for-service payments, and other functions (see below). Patients pay for most ambulatory care and are reimbursed, in the proportion stipulated by law: for most consultations and most patients this proportion is 75 per cent, the remaining 25 per cent representing the co-payment. On the other hand, the bulk of hospital costs is paid directly by the *mutualités*, with only a token co-payment by patients.[44] There is free choice of doctors, and patients do not need to see a general practitioner before consulting a specialist – *i.e.* there is no "gatekeeper". The system is subject to extensive regulation by both the authorities (federal and regional) and the *mutualités* and provider organisations themselves (through self-regulation). The central government has the final responsibility for the level of contributions and reimbursement rates, as well as for the rules governing the relationship between the *mutualités* and the providers. Moreover, it has veto power over the level of fees, and plays a central role in negotiating drug prices. It is also responsible for the accreditation of doctors and nurses, and regulates the hospital sector (see below).

Since co-payment by patients represents only around 15 per cent of total costs, the main source of financing health care covered by compulsory insurance is public revenues, with social security contributions and general taxation accounting each for a little over 40 per cent, and some other minor sources – such as contributions from old-age pensions and taxes on car insurance – accounting for the remainder. With effect from 1995, however, the "*gestion globale*" (global management) of the social security system abolished the distinction between contributions and tax revenues for each branch of the social security taken separately. There are no longer, therefore, any specific contributions for health insurance, since this scheme is an integral part of the social security system.

In addition to compulsory health insurance, the *mutualités* offer voluntary health insurance for minor risks (out-patient care, etc.) for the self-employed, about 75 per cent of whom have such an insurance. The *mutualités* also offer

complementary insurance to cover additional services for all of their affiliated members. This last segment of the insurance market has grown steadily over the past few years – from BF 8.3 billion in 1993 to BF 10.7 billion in 1996. Risk-based private health insurance offered by private for-profit companies is still very small compared with public health insurance but, like complementary insurance offered by the *mutualités*, it has grown steadily, especially for hospital costs, apparently reflecting some reduction in coverage and reimbursement in the compulsory scheme. Unlike *mutualités*, which are prevented by law from doing so, private insurance companies can offer cover for the proportion of medical expenses which the individual must pay under the compulsory insurance.[45] But apart from special situations or market niches, the possibility for private for-profit insurance companies to compete with *mutualités* on the market for complementary health insurance seems limited, given the tax-exempted status of the *mutualités* and their possibility of relying on economies of scale; and private for-profit insurance companies are excluded by law from the compulsory health insurance market. The turnover of private for-profit insurance companies has grown from BF 4.3 billion in 1993 to BF 6.2 billion in 1996. By comparison, outlays of the compulsory and voluntary public insurance schemes on account of current health expenditures amounted to over BF 440 billion in 1996.

The mutualités

The *mutualités* are organised mainly according to religious or political affiliations – a situation reflecting historical circumstances.[46] Membership in a *mutualité* is compulsory for most employees, self-employed and retired people, but the choice of *mutualité* is free. However, with the compulsory insurance cover offered and the contribution rates levied being determined by law – and hence identical – the *mutualités* have essentially a captive and non-contestable market in this segment of health insurance, and competition for new members centres on supplementary programmes, on geographical convenience, and on the quality of service delivery, such as the speed of settling claims. Nonetheless, competition is strong, partly because revenue to cover administrative costs depends on the size of the membership. As for the financing of the insurance cost (as distinct from the administrative cost) of the *mutualités*, to preserve solidarity, surpluses and deficits are effectively pooled by a special government agency responsible for health and disability insurance (*Institut national d'assurance maladie-invalidité* or INAMI).[47] In fact, reflecting different revenue and risk levels of their respective memberships, *mutualités* have significantly different capacities to earn contributions as well as different claim burdens. Deficits usually exceed surpluses and, by *ad hoc* government decisions, the net position is generally financed through subsidies. Over the years, cumulated deficits have become much larger than cumulated surpluses. While this approach provides little incentive for the *mutualités* to reduce overall costs, it explains why overt "cream skimming" – *i.e.* the selection of low-risk and

high-income members by the *mutualités* – is largely non-existent. In addition to receiving their funds from it, the *mutualités* are also supervised by INAMI, which is itself administered by a general board composed of representatives of employers, employees, *mutualités*, and the government – with the latter having a power of veto over decisions. Health service providers are represented in an advisory capacity.

The fee schedule – the so-called "nomenclature" – on which reimbursement to patients is based, is negotiated each year in a system of medical *concertation* between representatives of the *mutualités* and the medical profession (*Commission médico-mutualiste*). In this case, as when bargaining with drug companies on pharmaceutical prices, the *mutualités* see themselves as representing the consumers. Since agreements must be approved by the government, this arrangement amounts to a bilateral monopoly supervised by the government. If more than 40 per cent of practitioners reject it, an agreement is not implemented, in which case the government has three possibilities:

- submit an alternative draft agreement to the practitioners;
- fix the fee levels for some or all of the services;
- fix the reimbursement levels, leaving the practitioners free to fix their own fees.

Reimbursement is generally 75 per cent of the negotiated fee,[48] except for the poor, the elderly and certain other social groups for which, as noted, above a certain yearly ceiling it is 100 per cent. Costs of hospital stay are fully covered by the compulsory national insurance (apart from a fixed and relatively modest daily co-payment for the part covered by insurance), with no distinction between private and public hospitals. However, costs of hospital stay and nursing care are strictly separated from costs of medical services (including laboratory and radiology services) which are covered by the negotiated fee schedule, with patients paying the co-insurance contributions.

Health care providers

Most physicians are independent and, with a few exceptions, paid on a fee-for-service basis. However, as noted, reimbursable fees are largely controlled by the government and set at the national level ("nomenclature"). Hence, competition among physicians is mainly of a non-price character. The "nomenclature" sets an effective price floor. The Order of Physicians considers pricing below the negotiated level unethical practice, and may even temporarily suspend non-abiding physicians from medical practice. On the other hand, there are no legal or "ethical" impediments to charging fees above the reimbursable level ("nomenclature"), and this is not unusual, especially for specialists and specialist hospital services (Nonneman and van Doorslaer, 1994), although it may not always be done in an entirely transparent way.

About 60 per cent of all Belgian hospitals are non-profit private institutions, and the remainder are public institutions.[49] Most of the private hospitals are owned by non-profit associations (ASBL – *Association Sans But Lucratif*) which originally had links with religious charitable orders, some 5 per cent are owned by the *mutualités*, and a small number by doctors (specialists). Entry into the market for hospital services is restricted by government regulation. To be reimbursed by the *mutualités* and subsidised by the Department of Public Health, a hospital must obtain a license to operate a certain number of beds in each specific category (intensive care, surgery, maternity, etc.). For the financing of their activities, hospitals rely on two major sources: first a prospective budget based on a standard cost per day[50] set annually for each hospital by the Department of Public Health; and second, fee-splitting arrangements with staff physicians. The standard cost per day is also the price per patient-day; it is paid directly to the hospital by the patient's *mutualité* which is itself 75 per cent financed by the INAMI and 25 per cent by the Department of Public Health. In addition, the patient pays to the hospital a fixed sum per day as a co-insurance contribution. The budget system is designed to contain hospitals' overall costs and promote efficiency (mainly through the calculation of the standard cost per day which, as noted above, is partly based on average costs of a representative sample of hospitals). Measures have been taken over the last few years to ensure a better division of resources between hospitals.

In addition to the licensing system, the government – in co-operation with Regional and Local authorities – controls overall hospital capacity through the financing of investment in this sector. The rules applying to the financing of investment are broadly the same for private and public hospitals.[51] In both cases, construction and renovation work has to be agreed by the Regional Ministry of Health, in which case the regional government, through a public subsidy, funds 60 per cent of the investment cost of public and private hospitals, with the remainder being funded by government-guaranteed loans.[52] Over the years, the government has used its control over the hospital sector, among other goals, to try and reduce the number of hospital beds, transforming some of them into less expensive nursing home beds.

As for the provision of longer-term medical care, the *mutualités* cover the cost of nursing care and help with daily activities in rest and nursing homes. They also cover the cost of care for persons who have been declared eligible for admission into homes for the elderly. In both cases, there are five categories of daily rates, depending on the degree of dependency. However, daily rates are somewhat lower for homes for the elderly.

Pharmaceuticals

Rules and regulations concerning the distribution of pharmaceuticals and the pharmaceutical industry are the responsibility of the federal government.

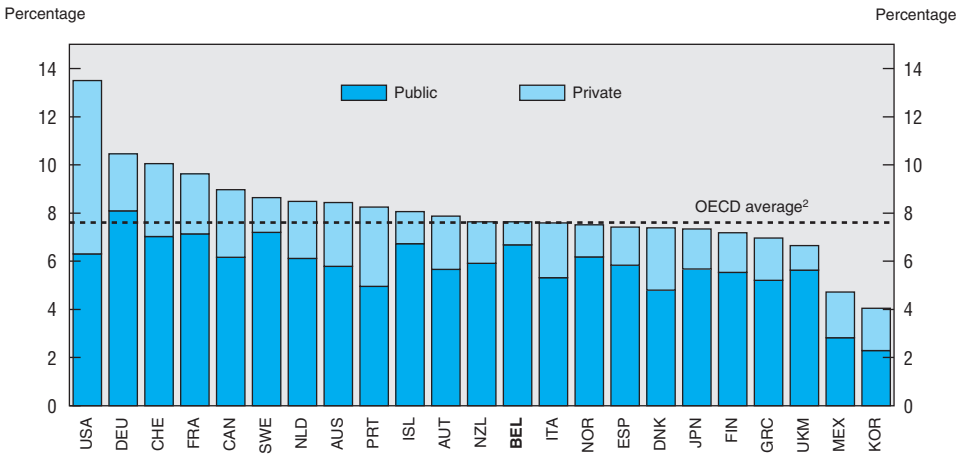
Hence, before their introduction on the Belgian market, the price of all drugs is set by the Ministry of Economic Affairs after consulting the Price Commission, on which are represented the social partners. Belgium is one of the last countries in Europe, with Luxembourg, with such an itemised price fixing. In addition, for drugs approved for reimbursement – *i.e.* meeting specific social and medical criteria – the price is also reviewed by a technical committee of the INAMI and made known to the pharmaceutical firm. When agreement has been reached on the price, the latter becomes a legal provision. For reimbursement purposes, drugs are divided into six categories. Drugs for the most serious pathologies are fully reimbursed: others have reimbursement rates of 75, 50, 40, 20 and 0 per cent.

Performance and costs

Health care spending[53]

Health care spending in Belgium is closely in line with that of other Member countries on average. Total health spending (around BF 660 billion) accounted for 7.6 per cent of GDP in 1997, exactly the same as the (unweighted) OECD average and very close to the EU average (7.9 per cent of GDP) (Figure 11). At around $1 700 (measured at purchasing power parity) per capita health expenditure was only marginally above the OECD average, and was close to the

Figure 11. **Health expenditures in OECD countries**
As a percentage of GDP, 1997[1]

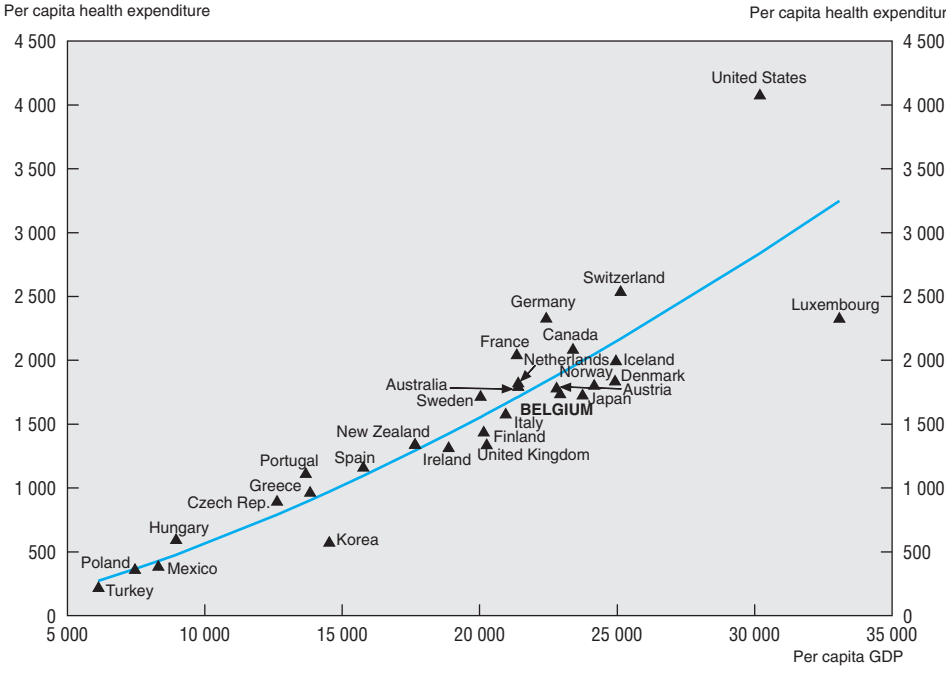


1. Total expenditure.
2. Unweighted average, 1996 for Turkey.
Source: OECD Health Data 98.

"expected" level, given Belgium's per capita income (Figure 12). On the other hand, Belgium has the smallest proportion of private health expenditure, which in 1997 represented only a little over 12 per cent of total health expenditure (or about 1 per cent of GDP).

Over the past few decades, nominal health spending (including investment) has grown very rapidly in Belgium, generally exceeding GDP growth by a significant margin. But this margin has varied considerably from decade to decade: after being rather narrow in the 1960s, especially compared with the OECD average, it widened sharply in the 1970s, before shrinking again in the 1980s and in 1990-97, when – at 0.3 percentage point – it was well below the OECD and EU averages (Table 6). Excluding investment, it is mainly expenditure for hospital

Figure 12. **Health expenditures and GDP per capita**
1997,[1] US$[2]



1. 1996 for Czech Republic, Hungary, Poland and Turkey.
2. Total expenditure on health care and GDP figures converted into US dollars at purchasing power parity exchange rates.
The equation of the regression line is the following:
LN (health expenditure per capita) = -7.17 + 1.47*LN (GDP per capita)
R squared = 0.92 T: (-8.75) (17.54)

Source: OECD Health Data 98.

Table 6. **The growth of nominal health spending**[1]

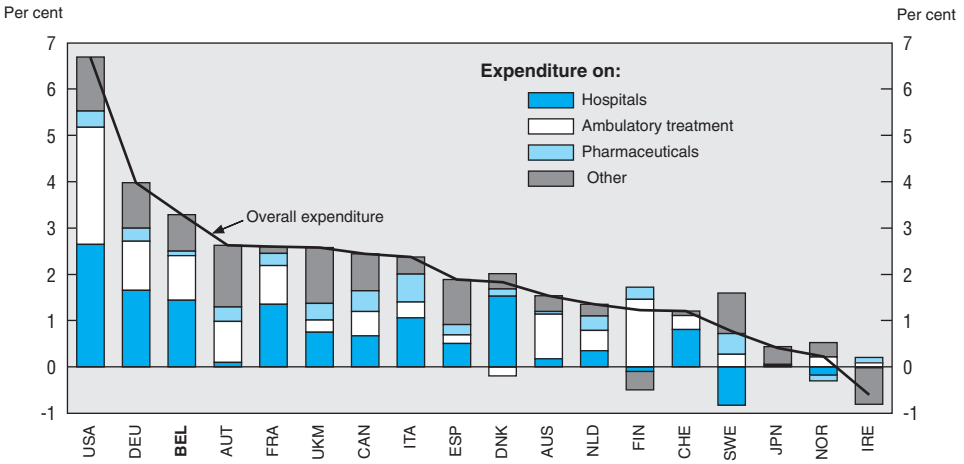
	Annual average growth in excess of GDP				As a percentage of GDP	
	1960-70[2]	1970-80[2]	1980-90[2]	1990-97[2]	1960[2]	1997[2]
Belgium	**1.9**	**5.4**	**1.5**	**0.3**	**3.4**	**7.6**
United States	3.6	2.5	3.5	1.6	5.1	13.6
Japan	5.0	4.1	–0.8	2.8	3.0	7.3
Germany	2.9	3.8	–0.1	2.7	4.8	10.5
France	3.6	3.0	1.8	1.6	4.2	9.9
Italy	4.0	3.7	1.7	–1.0	3.6	7.6
United Kingdom	1.5	2.6	0.7	1.6	3.9	6.7
Canada	2.8	0.3	2.6	–0.1	5.4	9.0
Australia	1.9	3.0	0.8	0.3	5.0	8.5
Austria	2.3	4.2	–0.7	1.4	4.3	7.9
Czech Republic	..	..	..	6.5	5.0	7.0
Denmark	5.6	4.2	–0.7	–1.1	3.6	7.4
Finland	4.2	1.5	2.3	–1.5	3.9	7.2
Greece	3.8	0.9	2.0	8.7	2.4	7.0
Hungary	..	..	..	–0.2	6.6	6.5
Iceland	4.9	3.2	3.5	0.2	3.3	8.1
Ireland	3.8	6.0	–2.9	0.1	3.8	6.8
Korea	..	4.4	3.4	0.6	2.1	4.0
Luxembourg	..	5.8	0.6	1.0	3.7	7.0
Mexico	..	..	..	5.4	3.5	4.7
Netherlands	4.9	3.3	0.5	0.4	3.8	8.5
New Zealand	2.2	1.8	1.2	1.3	4.4	7.6
Norway	4.6	5.2	1.2	–0.6	2.9	7.5
Poland	..	..	..	3.2	4.4	5.2
Portugal	..	9.4	1.5	3.6	2.8	8.3
Spain	10.5	5.1	2.3	1.1	1.6	7.4
Sweden	4.8	3.0	–0.7	–0.3	4.7	8.6
Switzerland	5.1	3.5	2.0	3.0	3.2	10.1
Turkey	..	4.5	1.3	1.1	2.4	3.8
EU[3]	4.1	4.1	0.6	1.2	3.6	7.9
OECD[3]	4.0	3.8	1.1	1.5	3.8	7.6

1. Including investment.
2. Or nearest year available.
3. Unweighted average.

Source: OECD *Health Data* 98.

and ambulatory care which has accounted for the rapid long-term growth of total health expenditure. Expenditure for pharmaceuticals, on average, has grown at a more moderate pace, and as a proportion of GDP has recorded little net change since 1970 (Figure 13). Restricting the analysis to the last 15 years and to current health expenditure of the public sector,[54] the share of hospital fees has declined

Figure 13. **Growth in health spending by category**[1]
Change in ratio to trend GDP (percentage points) 1970-97[2]



1. Current expenditure. For Ireland, Sweden and United Kingdom, public expenditure is used.
2. Or closest year available.
Source: OECD Health Data 98.

somewhat – from 28½ per cent of total expenditure in 1980 to 25½ per cent in 1995 – and the share of practitioners' fees (including clinical biology) has fallen more markedly – from 43 per cent to 33 per cent, with half the decline accounted for by a fall in the expenditure share of clinical biology. Over the same period, the share of expenditure for pharmaceuticals has first declined and then risen, showing virtually no net change (16½ per cent of the total); and the share of all other current health expenditure has risen from 12 to 25 per cent, partly reflecting rapidly growing expenditure for advanced medical technologies. As in several other OECD countries, these expenditure trends seem to reflect, at least in part, a substitution towards less costly components of health care in response to spending controls or constraints imposed on hospitals and certain medical acts, such as clinical biology. Indeed, in line with tendencies in other OECD countries, the average length of hospital stay has dropped – from nearly 21 days in 1977 to 11 days in 1996 – and both the number of hospital beds per inhabitant and their average occupancy rate have declined (Table 7).

The rapid growth in health care expenditure since 1970 has reflected many factors. Some of them – such as the progressive ageing of the population, the increase in the standard of living combined with the desire for better health care, the continuing development of expensive new medical techniques, and a more qualified and better paid medical staff – have been common to several

Table 7. **Hospital capacity and utilisation**

	Beds for 1 000 inhabitants		Average bed occupancy (per cent)		Average length of hospital stay (days)		Admission rate of population (per cent)	
	1970[1]	1996[1]	1970[1]	1996[1]	1970[1]	1996[1]	1970[1]	1996[1]
Belgium	**8.3**	**7.2**	**85.7**	**83.6**	**20.7**	**11.3**	**9.3**	**20.0**
United States	7.9	4.1	80.3	66.0	14.9	7.8	15.5	12.4
Japan	12.7	16.2	81.6	83.6	55.3	43.7	5.4	9.3
Germany	8.8	9.6	88.6	80.9	23.7	14.3	15.4	20.9
France	9.2	8.7	83.2	81.1	18.3	11.2	7.4	22.7
Italy	10.5	6.0	77.9	72.0	19.1	9.8	15.7	16.2
United Kingdom	9.4	4.5	82.1	80.6	25.7	9.8	10.9	23.1
Canada	7.0	5.1	80.4	84.2	11.5	12.0	16.5	12.5
Australia	11.4	8.7	83.0	82.9	. .	15.5	. .	16.9
Austria	10.8	9.2	86.4	78.8	22.2	10.5	15.5	25.1
Czech Republic	11.4	9.5	79.3	77.6	16.3	12.8	20.2	21.0
Denmark	8.1	4.9	80.6	81.3	18.1	7.3	14.4	19.8
Finland	15.1	9.2	91.0	87.7	24.4	11.6	18.2	25.7
Greece	6.2	5.0	76.0	69.4	15.0	8.2	10.5	15.0
Hungary	8.1	9.3	89.8	76.5	15.4	10.8	16.6	23.4
Iceland	13.0	14.8	98.3	84.0	28.3	16.8	16.4	28.0
Ireland	5.5	3.7	80.1	83.2	13.3	7.2	12.4	15.5
Korea	1.6	4.6	56.6	69.6	14.0	13.0	0.8	6.3
Luxembourg	12.6	10.7	82.6	75.0	27.0	15.3	13.4	19.4
Mexico	0.9	1.1	52.9	68.6	. .	4.1	. .	5.8
Netherlands	11.4	11.2	90.9	88.7	38.2	32.5	10.0	11.1
New Zealand	10.8	6.8	. .	57.3	15.8	6.5	9.3	13.8
Norway	15.7	15.0	83.1	82.2	21.0	9.9	13.2	15.3
Poland	5.2	5.5	. .	. .	. .	11.0	10.1	11.6
Portugal	6.3	4.1	74.1	73.9	23.8	9.8	6.9	11.4
Spain	4.7	4.0	69.0	76.7	18.0	11.0	7.1	10.0
Sweden	15.3	5.6	83.6	81.9	27.2	7.5	16.6	18.1
Switzerland	. .	20.6	84.6	82.6	26.0	25.2	13.1	15.0
Turkey	2.0	2.5	52.0	57.4	9.0	6.3	4.2	6.3
EU[2]	9.5	6.9	82.1	79.7	22.3	11.8	12.2	18.3
OECD[2]	8.9	7.8	79.8	77.4	21.6	12.9	12.0	16.3

1. Or nearest year available. For Belgium, data refer to, respectively: 1970 and 1996; 1977 and 1995; 1977 and 1996; and 1970 and 1996.
2. Unweighted average.

Source: OECD *Health Data* 98.

OECD countries; but some other factors have been more specific to the Belgian health care system. In fact, especially until a few years ago, distorted incentives insufficiently countered by regulations seem to have encouraged excess supply

conditions and over-consumption in various segments of the health care market. Even today, despite an increase in the co-payment for ambulatory care, patients may still have insufficient incentive to limit their demand; and since providers, with only a few albeit significant exceptions, are paid on a fee-for-service basis, they have strong incentives to expand service and induce demand. At the same time, the *mutualités* and the government have been rather successful in keeping medical fees and pharmaceutical prices relatively low by international standards. Hence, it has been said that Belgium has a "low price-high volume health system" (Nonneman and van Doorslaer, *op. cit.*, p. 1487).

Regional differences in medical consumption and expenditures per capita seem to be large in Belgium, even compared with a theoretical norm, *i.e.* after allowing for differences in patient characteristics such as age, income, labour market status, education and environmental factors (Schokkaert, Van Dongen and Dhaene, 1991). However, the latest report of the *Conseil général* of the INAMI (INAMI, 1996), notes that over the recent period there has been a convergence in the evolution of the average health cost per beneficiary in Flanders and Wallonia as compared with the national average. More importantly perhaps, the report stresses that differences in per capita medical consumption within regions are also very large and often larger than averages between regions.[55]

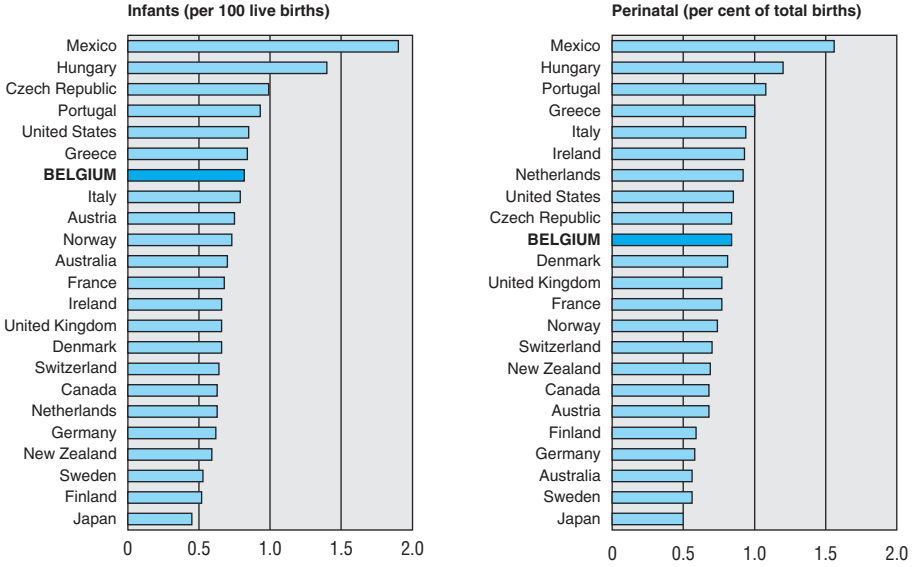
Performance

The relationship between spending on health care and the health status of the population is complex, being affected by many factors, some of them difficult to define and measure – *e.g.* life style. Moreover, the concept of health status is itself multi-dimensional and hard to quantify. Nonetheless, on the basis of various objective indicators, it would seem that the overall health status of the Belgian population has improved significantly over the past decade, although it remains about average compared with that of other Member countries.

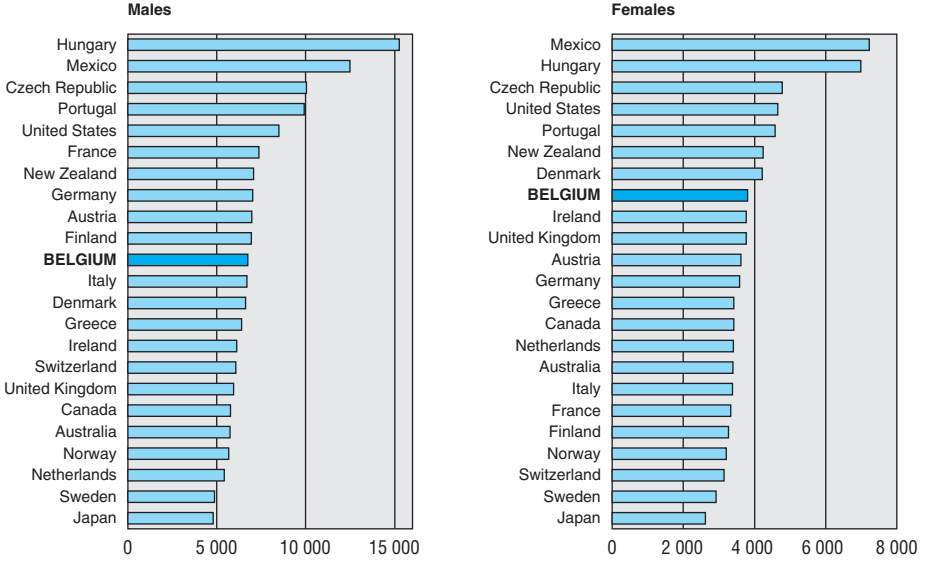
Life expectancy at birth – which was 70 years for men and 77 years for women in 1982 – rose to 74 and 80 years, respectively, in 1997, ranking among the highest in the OECD area. Moreover, life expectancy is still increasing, and the differential between males and females has narrowed. On the other hand, while infant mortality has declined from 1.6 per 100 live births in 1975 to 0.8 in 1992, it remains above that of most other OECD countries; and perinatal mortality – at 0.8 per cent of total births in 1992 – is only about average by international standards (Figure 14, Panel A). The same is true for the Belgian performance in terms of the number of avoidable years of life lost under age 70 per 100 000 persons – around 6 700 for males and 3 800 for females in 1992 (Figure 14, Panel B). This is a useful indicator of health status and of outcomes of the health system since it represents the shortening of life expectancy due to avoidable or curable diseases. Measures have been taken in this area, but it is still too soon to assess

Figure 14. **International comparison of health outcomes**

A. Child mortality rates, 1992



B. Potential years of lives lost, 1992 (per 100 000 persons aged under 70)



Source: OECD Health Data 98.

their effects. Finally, according to the authorities, broadly speaking, Belgium is not confronted with a problem of waiting lines for patients in the health sector: access to general practitioners seems to be particularly quick and convenient, and there are few people waiting for hospital in-patien care.[56] While the absence of queues is obviously a plus for patients, the broader assessment of this state of affairs is more complex. In a world with competitive markets and complete information, queues are unquestionably a sign of market distortions and inefficiency. But the market for medical services in Belgium is not competitive and, as elsewhere, consumers (patients) are inevitably less well-informed than doctors. Moreover, to the extent that public health insurance could lower the marginal price of medical services below the marginal social cost, with access to such services not being controlled by efficient mechanisms, the question might arise whether the absence of queues is not a sign of oversupply and over-consumption, rather than a sign of efficiency of the system.

Recent policy measures and developments

Even if there has been no overall reform and most of the many measures introduced have been taken on a step-by-step basis, Belgium's health care system has undergone considerable change since the mid-1980s. The first such changes concerned the hospital sector, with services reserved for acute and chronic cases and psychiatric beds being reorganised or rationalised, while the method of allocation of the hospital budget was fundamentally overhauled. Other specific measures were also taken. For instance, starting in 1985, attempts were made to better control what seemed to be an excessive use of diagnosis testing (clinical biology). A national budget for this item was introduced, combined with a data management system to identify high prescribers. In 1989, the fee-for-service system for hospital patients was replaced by a daily rate and a fee per admission; and in 1992, a new system for out-patient tests was introduced, with sharply reduced fees, strict monitoring of individual prescription behaviour, and the request that heavy prescribers provide an explanation. Also, in 1990, the government was allowed by Parliament to fix a global budget for health care expenditure with, in the case of target overruns, obligatory corrective mechanisms – such as automatic downward fee adjustment and obligatory refunding by high-cost providers. In practice, however, the automatic adjustments were not respected. This approach consisting of direct government control in the form of fixed budgets and sanctions brought expenditure in this area reasonably under control: gross inefficiencies by over-prescribing for certain special cases were curbed, if not eliminated (Nonneman and van Doorslaer, *op. cit.*, p. 1490).

The economic environment deteriorated seriously around 1993 and, with Belgium wanting to meet the criteria for membership of the European Monetary Union, the government introduced a ''global plan'' for employment,

competitiveness and the social security system. In this last area, confronted, on the one hand, with a new acceleration in the growth of spending and, on the other, with the imperative need to resume fiscal consolidation after an increase in the budget deficit during the economic downturn of 1993, the plan aimed at cutting social security expenditure and increasing revenues. As regards health care, corrective measures centred on the introduction of a "growth norm", limiting the annual increase in expenditure to 1.5 per cent in real terms, starting in 1995.[57] The "norm" determines a global budget for health care expenditure as well as budgets or targets for sub-sectors ("*enveloppes budgétaires*"). Its implementation is monitored closely – *i.e.* on a quarterly basis – with the automatic activation of corrective measures – such as the adjustment of fees and rates of reimbursement – in the case of target overruns. The "norm", however, allows a certain degree of flexibility: for instance, it excludes some exceptional or specific expenditures – such as increases in salaries granted to health care personnel, and epidemics – and the triggering of automatic corrective measures is not, in principle, aimed at ensuring the respect of the "norm" within the year itself, but generally for the following year, except in certain sectors.

In addition, the 1993 reform aimed at increasing the cost-consciousness and cost-participation (*responsabilisation*) as well as the supervision of all major players in order to modify their behaviour and reduce unnecessary consumption of medical acts and pharmaceuticals. In the case of service providers, this was to be achieved through the so-called "medical profiles" (*profils médicaux*) about services and prescriptions by individual doctors, so as to detect obvious abuse, and a more extensive use of the budget technique for clinical biology. Patients' behaviour was to be affected mainly through a selective increase in co-payments, and health funds (*mutualités*) were to be made more financially responsible for health expenditure. After long negotiations, in mid-1994 it was decided that starting in 1995, *mutualités* would be progressively more responsible for their budget shortfalls, in line with the following time schedule:[58]

1995-96	15 per cent of the shortfalls;
1997-98	20 per cent of the shortfalls;
from 1999	25 per cent of the shortfalls.

However, *mutualités* would not be responsible for shortfalls caused by "exogenous factors", such as lower income received from the social security sector, increases in health expenditure due to wage increases granted to nursing staff and epidemics. The *mutualités* were left free to decide how they would finance their share of any shortfalls, with an increase in contributions being considered as the most likely solution. As a result of these arrangements, *mutualités* were expected, in their bilateral negotiations, to press health providers to contain expenditure, thereby reducing the risk of budget shortfalls. That said, the cost participation of the *mutualités* is both partial and asymmetric.[59]

To mitigate the impact of the increase in co-payments and preserve as much as possible the principle of free access to health care to everyone, the government took a number of accompanying measures, including:

- a widening of the definition of especially vulnerable social groups – originally widows, invalids, pensioners and orphans (VIPO) whose incomes do not exceed a certain amount – enjoying special treatment concerning co-payments;
- the introduction of yearly co-payment ceilings for especially vulnerable social groups (*franchise sociale et fiscale*) above which patients pay no co-payments (see note 41).

Despite these corrective efforts, expenditure for public health increased rapidly in 1996, exceeding the "norm" by a large margin, partly as a result of accounting factors – a shift of some expenditure from 1995 to 1996 – and a flu epidemic. A number of new measures – some of them of a structural character – were thus taken in September and December 1996, as well as in March 1997 and in the 1998 budget. Most of these measures represented selective cuts in medical fees, tariffs and *forfaits* but they also included levies on pharmaceutical companies. Provisional figures show that the "norm" was respected in 1997, but a new overrun is expected in 1998 (with a deficit in the health insurance scheme of nearly BF 8 billion).

In early 1998, reacting to an initiative of the authorities to limit doctors' fee supplements in hospital wards with two or more beds, the medical profession, through one of its unions (Absym), declared the 1998 fee schedule no longer in force, thereby ending the so-called P*ax medica*. The fee schedule ("nomenclature") had been negotiated, as every year, by representatives of the medical profession and the *mutualités* (C*ommission médico-mutualiste*). This decision of the medical profession gave practitioners the possibility of setting their fees on an entirely free basis – albeit with an unchanged level of reimbursements. For the time being, however, the majority of members of the medical profession have not taken advantage of this possibility. While discussions and initiatives aiming at restoring the P*ax medica* have continued, the government has formed several working groups to examine the supply structure and interrelations within that structure in the health system.

The task ahead

Current and expected problems

The Belgian health care system, as already noted, has many positive features which make it especially attractive for patients. Patients are provided with a comprehensive solidarity-based insurance coverage, receive good service,

and are subject to only modest co-payments; they are also free to choose their doctors, as well as their insurer; and, under a largely fee-for-service system, they can demand almost any treatment. But these very features, combined with independent medical practice (*médecine libérale*), create a weak or distorted pattern of incentives which are not sufficiently countered by government control and self-regulation by the medical profession. This makes the system vulnerable to abuse and inefficiency, resulting in over-consumption and unnecessary expenditure.

According to the authorities, there is an over-supply of hospital beds, doctors, dentists, physiotherapists and pharmacists – a situation which may contribute to unnecessary medical acts and an over-consumption of pharmaceuticals. Available evidence largely confirms this assessment. At 3.4 per 1 000 inhabitants, the number of physicians in Belgium in 1995 was above the EU and OECD averages – 3.1 and 2.8, respectively – and the fifth highest in the OECD area (Table 8). Moreover, in the ambulatory sector, the number of contacts with physicians, at 8.0 (per capita and per year), was also high by international standards. As for the consumption of pharmaceuticals, expenditure per capita was among the highest in the OECD area in 1996, although prescriptions per capita were below the EU and OECD averages in 1993, and average hospital bed occupancy was above these averages in 1995. Finally, both the number of hospital beds per inhabitant and the average length of hospital stay were broadly in line with international standards (Table 7).

The structural shortcomings discussed above may help explain the difficulty encountered by the authorities in their efforts to restrain the growth of health spending. Combined with other factors, they make the outlook of the health care system especially uncertain. In fact, in coming years, health expenditure can be expected to continue to be buoyed by several factors, including: the desire for better health care partly related to a rising standard of living;[60] the cost of more highly qualified medical personnel; and the introduction of new expensive medical technology. Moreover, the impact of population ageing is projected to increase, accentuating the demand for medical care (Lambrecht, 1997; Fasquelle and Weemaes, 1997).

At the same time, the authorities see little likelihood of revenue rising more than as a result of the positive effects deriving from economic growth and higher employment: on the one hand, there is a pressing need to pursue fiscal consolidation and rapidly reduce the debt-to-GDP ratio (see Chapter II); on the other, the government wants to cut social security contributions further to reduce labour costs and promote employment of low-skilled workers. Since "alternative financing" – *i.e.* taxes and other revenues which do not directly affect the cost of labour – can hardly be expected to provide sufficient additional revenues, the equilibrium of the social security system could mean either allocating the savings made in other social security sectors or in interest payments to the health care

Table 8. **Indicators of resource use in the health sector**

	Average growth (number of physicians)		Physicians per 1 000 inhabitants	Proportion of specialists	Ambulatory sector		
					Contacts with physicians, per capita per year	Prescriptions per capita	Total expenditure on pharmaceuticals per capita[2]
	1970-80[1]	1980-96[1]	1996[1]	1996[1]	1996[1]	1996[1]	1996[1]
Belgium	**4.8**	**2.8**	**3.4**	**46.9**	**8.0**	**10.4**	**306**
United States	3.6	2.8	2.6	50.0	6.0	6.5	344
Japan	2.8	2.9	1.8	. .	15.8	. .	349
Germany	3.4	4.4	3.4	62.6	6.4	13.5	289
France	5.2	3.1	2.9	50.2	6.3	52.0	337
Italy	9.8	4.8	5.5	. .	11.0	21.1	284
United Kingdom	2.7	1.7	1.6	. .	5.9	9.3	218
Canada	3.6	2.2	2.1	42.4	6.8	16.1	258
Australia	4.9	3.5	2.5	36.4	6.6	9.6	202
Austria	2.0	3.8	2.8	54.4	6.3	17.2	247
Czech Republic	2.7	1.5	2.9	74.4	. .	32.1	234
Denmark	4.9	2.2	2.9	. .	5.3	7.4	165
Finland	6.8	3.6	2.9	56.3	4.3	6.3	209
Greece	5.1	3.8	3.9	55.7	5.3	9.0	236
Hungary	4.0	2.2	4.2	64.4	14.8	28.6	172
Iceland	5.3	3.6	3.0	. .	4.8	16.4	312
Ireland	2.5	3.4	2.1	. .	6.6	13.5	126
Korea	. .	7.0	1.2	63.2	9.5	. .	. .
Luxembourg	4.9	2.6	2.2	64.4	. .	14.5	250
Mexico	. .	5.5	1.5	45.8	2.2	. .	32
Netherlands	5.2	3.4	2.6	35.8	5.4	8.0	193
New Zealand	5.3	2.8	2.1	30.4	3.8	8.8	214
Norway	4.2	2.8	2.8	62.9	3.8	6.9	174
Poland	3.2	2.3	2.4	. .	5.4	. .	. .
Portugal	9.0	2.8	3.0	68.6	3.2	19.8	282
Spain	6.6	4.2	4.2	. .	6.2	26.5	223
Sweden	5.7	2.5	3.1	71.1	2.9	6.4	218
Switzerland	5.4	2.6	3.2	35.7	11.0	19.0	190
Turkey	7.0	6.5	1.1	42.1	1.0	. .	60
EU[3]	5.2	3.3	3.1	56.6	5.9	15.7	239
OECD[3]	4.8	3.3	2.8	53.0	6.5	15.8	227

1. Or nearest year available. For Belgium, date refer to, respectively: 1971-80; 1980-95; 1995; 1995; 1993; 1993 and 1996.
2. Figures converted into US dollars at purchasing power parity exchange rates.
3. Unweighted average.

Source: OECD *Health Data* 98.

sector, or reducing expenditure. Raising patients' co-payments could reduce health expenditure or, at least the financing need of the compulsory health insurance scheme. But here too the authorities believe that the room for

manoeuvre is very limited: international evidence shows that the demand for medical acts is generally rather price inelastic; and, for equity reasons, an increase in co-payments would, no doubt, be combined with extensive exceptions for vulnerable social groups.

Finally, the "growth norm" cannot be a permanent instrument of health care cost control since it has little if any relation to any notion of efficiency or optimality. It was introduced as a temporary measure to curb health expenditure and ensure further fiscal consolidation. It needs to be replaced by measures based more on cost benefit analysis or appropriate incentives.

The need for further initiatives

Without the "norm" and with the spontaneous trend growth of expenditure likely to exceed that of revenues, the health care system in its present form may not be sustainable over the longer term. But the degree of uncertainty is very high. In addition to the variables already mentioned, the outcome will also depend on a number of more general considerations, including: the overall economic context and particularly the rate of growth of real GDP; progress in correcting the employment/unemployment problem which could have a major impact on the financial position of the social security system; and socio-political decisions concerning the share of social security expenditure or even total public expenditure to be allocated to the health sector. Without taking a firm view on any of these points or on any specific scenarios, it would seem prudent to prepare for a progressive squeeze of resources in the health sector, and take appropriate structural measures to curb expenditure, especially in view of the desirability of dropping the "growth norm".

The authorities broadly agree on the need for reforming the health system but, given the special characteristics of health care – notably the difficulty that consumers have in making informed choices – and country-specific considerations, they strongly favour remaining within the existing framework of largely non-market arrangements, continuing to rely on *concertation* among major players, under the supervision and ultimate management of the government. The stated aim is to ensure that the Belgian health care system will continue to reconcile the principles of solidarity, quality of care, and freedom of choice at a reasonable cost (compared with other OECD countries). For the time being, the authorities envisage continuing with the recent approach, implementing gradual changes by expanding or complementing initiatives which have already been introduced. This approach largely relies on: the *responsabilisation* of all the major players with respect to the cost of health care; "peer pressure" on doctors and other care providers; and a large number of rules and regulations to curb expenditure – including stricter controls on the supply of hospital beds, doctors, dentists and certain other care providers, and a wider use of *forfaits* (*i.e.* fixed amounts) for

medical institutions, single patients and specific medical acts. To foster progress in all these directions, the authorities count on closer collaboration between various relevant institutions – notably between the Public Health Administration (*Administration de la santé publique*) and the INAMI – as well as on better information and statistical data – hence their commitment to develop PHARMANET and other medical data banks, and to introduce a "*dossier médical centralisé*" (central medical file).

Through rules and regulations, the focus is on limiting as much as possible inefficiencies resulting from the distorted pattern of incentives implicit in the basic institutional structure of the system; relatively little is planned to modify these incentives and the structure of the system through the injection of more competition or other market forces – such as a further increase in co-payments. The authorities and apparently also a large majority of the Belgian "establishment", if not of the population at large, consider managed care – especially in the form implemented in the United States by for-profit health-maintenance organisations (HMOs) – to be alien to Belgian aspirations and inconsistent with the existing institutional framework. Yet, over the past few years, the Belgian health care system has arguably moved in the direction of a form of managed care, albeit an opaque one, based on solidarity, with a national pooling of risk and under the ultimate management of the government. In fact, as noted, the aim of the norm is to determine the total amount to be spent by the public sector on health care *ex ante* on the basis of resources available, rather than *ex post* on the basis of perceived need and actual expenditure; and the allocation of this predetermined total amount is done using techniques such as budgets for specific health institutions (*e.g.* hospitals and nursing homes), *forfaits* for special diseases or medical acts, and the requirement that to be sure of reimbursement patients obtain the permission (from their *mutualité*) before undergoing an expensive operation or treatment.[61]

Some of the corrective measures currently considered by the authorities – notably a wider use of *forfaits*, reliance on "peer reviews", and the introduction of a "*dossier médical centralisé*"[62] – seem promising or, at least, may be worth exploring. Through a reduction of unnecessary medical acts, they could lead to significant savings. Other measures – such as better information and the *responsabilisation* of all parties – may also be tried but, unless they are translated into concrete guidelines or cost/price incentives, can hardly be seen as more than ancillary initiatives. Considering the current approach as a whole, it remains to be seen whether, over the longer term, it will be sufficient to broadly equate expenditure and revenue. The outcome will depend, among other factors, on difficult-to-quantify technical and medical considerations – such as the scope for a wider use of "*forfaits*" for specific pathologies or medical acts, and the related potential savings. It is also uncertain whether the Belgian system of *concertation*, which worked reasonably well in the past when health expenditure was allowed to grow rapidly,

can be relied upon to strongly curb expenditure and allocate it among competing claims. While recent friction between doctors and *mutualités* and the end of the P*ax medica* may be just a temporary aberration, they may also be an indication of things to come.

Unfortunately, there is no theoretical reference model in the realm of health care, in part because one feature of the market is that consumers (patients) are imperfectly informed. In practice, models are fairly different from one country to another, and it would be dangerous to conclude that techniques that seem to work relatively well in one country could be used just as successfully in others. Market-oriented measures are no panacea in this area; admittedly, they are often used to curb the growth of health care expenditure, but they may conflict with the principle of equity or equality of treatment, and even their effectiveness is not always proven. However, the Belgian authorities might envisage taking market-oriented measures on a strictly limited basis, and only as a complement to their administrative system, taking care that this does not unduly weaken the principle of equity or equality of treatment in the public health care system. Of course, decisions on the efficiency/equity trade-off require value judgements which ultimately belong to the socio-political realm, rather than to economics. Nonetheless, if the sustainability of the Belgian health care system is in doubt and difficult decisions need to be taken, a more appropriate pattern of incentives could significantly facilitate the adjustment by reducing waste and inefficiency. Moreover, insofar as risk-based private insurance and *forfaits* already exist in the Belgian health care system, a further modest reduction of the set of medical services covered by the compulsory health insurance scheme – with greater latitude for additional coverage by *mutualités* or risk-based private insurance – combined with the introduction of capitation payments to curb the growth of spending under the existing fee-for-service system, would be more a question of degree than of principle.

To allow private for-profit insurers to provide an efficient complement to public compulsory insurance would require a level playing field in the provision of health care insurance, a situation which currently does not exist since the *mutualités*, which also offer supplementary health insurance, are not subject to various taxes paid by private insurance companies. Moreover, with regard to the provision of compulsory public insurance, it might be useful to consider whether *mutualités* should be allowed to compete on the basis of contribution rates – possibly with no right to reject applicants, in order to avoid "cream skimming". This approach may be more conducive to efficiency in the provision of medical care than the current system where, with the service package as well as contributions and reimbursement rates fixed and identical for all *mutualités*, the latter compete merely in terms of peripheral services for their members – such as holiday camps, etc. Moreover, to clarify their role, avoid conflicts of interest, and enhance competition, *mutualités* should no longer be allowed to act, directly or indirectly, as

providers of medical care through their ownership or control of hospitals and pharmacies, even though such activities may give them a better understanding of the area they are dealing with.

A limitation of patient's "free choice" – defined not just as the ability to choose their practitioner, but also to ask for virtually any kind of treatment – may be necessary to minimise excessive use of what appears to consumers to be a nearly free health service. This "moral hazard" issue is particularly important for a health insurance scheme relying on solidarity finance; and since the health care market is characterised by imperfect knowledge on the side of the customers (the patients), a limitation of freedom of choice may not necessarily entail a reduction in effectiveness and quality of treatment, or in welfare. To keep a balanced approach, such a limitation of choice may best be achieved by experimenting with a combination of new regulations and techniques, including: a more extensive use of *forfaits* (as also envisaged by the authorities); capitation fees; and, possibly, the "gatekeeper" technique. With respect to the latter, however, there are pros and cons, and the net effect may be small. On the one hand, the "gatekeeper" system may enhance efficiency by preventing patients from seeing the wrong specialist or from seeing a specialist unnecessarily; on the other hand, it may increase the number of overall contacts with doctors as well as expenditure, since to see a specialist a patient in this case has to see a generalist first.

A more extensive use of *forfaits* may require an increase in resources – in terms of staff and expertise – of relevant public bodies to allow them to provide the necessary guidelines and monitor their implementation. This is likely to be a demanding task since technological progress will probably require guidelines to be constantly reassessed and often modified. Moreover, a reliable and extensive medical data base is an essential precondition for the successful introduction of many non-market based initiatives to contain expenditure: for instance, by allowing the identification of best or worst practice doctors and hospitals in an objective and quantified way, it would make "peer pressure" a significantly more effective tool; in the case of the capitation fee, it would minimise the risk that a patient's health profile and service needs might not be taken fully into account; and linking PHARMANET with a data bank of the prescription record of individual physicians should allow the system to reap the benefits of "best-practice" standards. The authorities feel that Belgian health data may soon be very detailed and comprehensive. But completing work on various data banks and linking them up in an operational system will require stepped-up co-operation between public institutions, hospitals and doctors. It might also raise questions concerning the threat to privacy and the risk of simplistic interpretations. While these concerns are legitimate and should be taken duly into account, they should not be allowed to prevent the introduction of these promising statistical tools. More generally, efforts should be stepped up to make an optimum use of information technology to process data about patients in order to establish which treatments work best and which are the most cost-effective.

A further significant restriction of the fee-for-service rule combined with "peer reviews" and other measures to limit consumption can be expected to lower the income of physicians, reduce their number over the long-term, and contribute to the correction of the current situation of excessive supply in this segment of the health care market. This would seem a more efficient way of tackling the problem, at least over the longer term, than rationing the number of doctors "allowed" to graduate from medical schools or limiting the number of physiotherapists to be *agréés* (certified) by the INAMI. This is the method introduced by the authorities who have, however, added a periodic assessment of the results achieved. If, as a temporary measure, a form of quantity control were deemed necessary, it might be preferable to impose a quota on access to medical schools (*numerus clausus*), rather than on graduation.

The consumption of generic drugs currently represents a very small proportion – less than 1 per cent – of total drug consumption. But the price gap between brand name drugs (protected by copyright) and generic drugs is also rather small – some 15-20 per cent – and concerns only about 40 products. It is, however, expected to widen significantly in a few years' time as a number of important and expensive brand name drugs will move to the generic category. Hence, although the narrowness of the Belgian market may make it less attractive to the producers of generic drugs, it might be appropriate to start encouraging the use of generic as opposed to brand name drugs – perhaps through different reimbursement rates and incentives for pharmacies to substitute generic for brand name drugs.

Finally, the authorities should consider stepping up preventive medicine, especially in the light of the findings of health surveys which ought to be conducted at regular intervals, and better co-ordinating it among the various levels of government concerned – even though results and payoffs in this area cannot be expected before several years. A recent study[63] has noted with concern the increased consumption of cigarettes and alcoholic beverages among the young (15-19 year olds) in Belgium. These are just two of the most obvious examples where additional and more effective preventive measures would be desirable: they could include raising, so far as is possible, taxation on tobacco and alcohol so as to make their prices more of a disincentive. But preventive policies should be reconsidered more generally, for instance with respect to vaccination, the screening of diseases, health education of the population and, last but not least, the use of car seat-belts.

In sum, the authorities agree that the health care system needs further reforming: first, to eliminate over-consumption and inefficiency; and second, over the longer term, to curb the growth of expenditure which would otherwise tend to outrun the growth of resources. They also acknowledge that the "norm" should be phased out. But rather than turning to market mechanisms, they envisage

pursuing the current approach, which includes: making patients more cost-conscious, relying on "peer pressure" on practitioners, and extending the use of various forms of *forfaits*, all in a framework of *concertation* among the major players and under the ultimate supervision of the government. In other words, rather than allowing the – perhaps in the long term inevitable – reduction in patient free choice and fee-for-service rule to be brought about by market forces, Belgium hopes to develop an administrative mechanism which will allow it to ration and allocate medical services, while better preserving equity and solidarity than would be the case with market forces.

Notes

1. The rate of capacity utilisation in manufacturing reached the record level of 83.4 per cent in the first quarter of 1998.
2. To improve the investment climate, at end-1997, the government decided to apply the principle of "fiscal ruling" to investments of both domestic and foreign origin. A "fiscal ruling" is a prior agreement between the tax authorities and enterprises concerning the tax liabilities generated by their investments.
3. The VAT rate for houses and flats of less than 190 m^2 and 100 m^2, respectively, was reduced from 21 per cent to 12 per cent.
4. The fairly big swings in the inflation rate during the course of 1998 were partly due to a revision of the consumer price index at the beginning of 1998.
5. For a discussion of this law and its implications see Chapter III.
6. Number of hours per day. Source: U*nion professionnelle des entreprises de travail intérimaire*.
7. So far, only two enterprises have concluded a collective labour agreement with their personnel which includes a 32 hour working week for some of them. Because of improvements scheduled to be made to this scheme, the number of enterprises concerned can be expected to increase.
8. Based on the end-June figures.
9. Unemployed aged 50 years or over.
10. From 514 000 at end-September 1995 to 438 000 at end-September 1998.
11. From 73 000 at end-September 1995 to 133 000 at end-September 1998.
12. While itself not entirely satisfactory, this concept may provide useful additional information on the degree of under-utilisation of labour resources in the Belgian labour market. In fact, since the mid-1970s, a large number of people have joined a variety of government programmes which allow them to become inactive (and leave the labour force), to acquire the status of partially unemployed, or to be hired in special partly-subsidised jobs. In some respects, even broad unemployment may understate the true degree of under-utilisation of labour resources in Belgium, since it does not include people on social assistance not seeking a job and discouraged workers with no social benefits – *i.e.* workers who either leave the labour force in the face of poor job prospects or decide not to enter it. The number of people on social assistance (MINIMEX) has increased sharply since 1990 and represented some 1¾ per cent of the narrowly-defined labour force in 1997. Only about one-third of them are registered as job seekers and hence included in unemployment figures. As for discouraged workers, according to data compiled by the OECD Secretariat, they represented around 1½ per cent of the narrowly-defined labour force in 1993. Unfortunately, there are no accepted standard calculations of "broad unemployment" that would serve as a basis for international comparison.

13. The European Employment Observatory has recently developed its own version of "broad unemployment" (or "extended unemployment"), applying it to all EU countries (European Commission, 1998).

14. Reflecting the growing importance of part-time work, on a full-time basis "broad unemployment" actually edged up in 1997, before falling in 1998.

15. Defined as total employment in persons, as a per cent of the working-age population. The problem with this indicator is that it does not show the extent to which low employment is due to individual choices – that is, the preference between working and not working (or working only part-time) – or to market imperfections.

16. The current-account surplus of the Belgian-Luxembourg Economic Union was 5.6 per cent of GDP in 1997 and may have been of the same order of magnitude in 1998.

17. The impact of interest rate changes also depends on the reaction function of the public sector, by far the most important debtor in Belgian financial markets. For all these reasons, in the case of Belgium, the calculation of a formal "financial conditions indicator", as the OECD Secretariat has done for a few other Member countries, does not seem to be warranted.

18. In view of the introduction of EMU, the National Bank of Belgium has taken a number of steps concerning the instruments of monetary policy. For instance, since the European Central Bank plans to use reserve requirements, which is a completely new instrument for Belgian money market participants, the National Bank of Belgium has proposed to domestic credit institutions to participate in the last four months of 1998 in a transitional system of reserve requirements to prepare for the new monetary environment.

19. That is, without any increase in nominal terms in the annual transfer received from the Treasury.

20. The saving was 1 per cent of GDP in 1992, 2½ per cent in 1993, 1 per cent in 1994, ¼ per cent in 1995 and 1½ per cent in 1996.

21. The Belgian definition of "primary surplus" is slightly different from the one used by the OECD Secretariat in that, while it does exclude interest paid, it does not exclude interest and other revenues received, with the result that the primary surplus thus calculated is normally bigger than when calculated by the OECD Secretariat.

22. Since the measures taken in the framework of the Convergence Plan 1997-2000 were largely structural, their positive impact on the budget is expected to decline only moderately in the next few years – from BF 100.2 billion in 1997 to BF 87.5 billion in 2000 (for the *Entité* I).

23. As a result of better-than-expected macroeconomic conditions and budget outcomes, some one-off corrective measures included in the 1997 budget – such as sales of public buildings – were not carried out.

24. For a more detailed analysis of the increase in the primary surplus since the early 1990s, see the 1997 *Survey*, pp. 41-43.

25. The increase in receipts has not, however, entirely offset the loss noted in earlier years.

26. The proceeds of privatisation approached 1 per cent of GDP in 1997. They included: BF 30 billion resulting from the disposal of 24.7 per cent of the capital of the ASLK/CGER; BF 54 billion resulting from the disposal of 50 per cent of the shares of

the holding *Crédit communal*/Dexia; and BF 6.7 billion resulting from the listing on the stock market of 14.8 per cent of the capital of the holding GIMV (*Société régionale d'investissement de Flandre*).

27. The proceeds were used, with the consent of the European Monetary Institute, to reduce foreign currency denominated government debt.
28. These assumptions include: an estimated underlying growth rate of the Belgian economy of 2.3 per cent; an inflation rate of 2 per cent; and an implicit interest rate on the public debt of 6 per cent.
29. These included: certain forms of voluntary work-sharing – such as part-time pre-retirement; "opening clauses" allowing firms to negotiate with their work force to pay below the wage minima set in collective contracts at the branch level; and the generalisation of exceptions for the statutory minimum wage.
30. An overview of progress in structural reform in a summary form is presented in Box 2.
31. In 1975, the social partners decided to introduce a guaranteed minimum monthly income (*Revenu minimum mensuel moyen garanti* or RMMMG) which is binding and in many respects equivalent to a legal minimum wage.
32. For instance, in the case of *le stage des jeunes*, *le contrat de première expérience professionnelle* and *l'emploi-tremplin*.
33. The law was introduced to simultaneously support employment and preserve international competitiveness – especially in the period following the end of the 1995-96 real wage freeze and leading to the introduction of EMU.
34. The law states that if, as a result of the automatic wage indexation (based on the "health index") plus the autonomous wage drift, labour costs were to increase more than the maximum allowed, the disposable margin for wage increases in the following years would be correspondingly reduced.
35. The *Maribel* measures were amended with effect from 1 July 1997, this resulting in the abolition of the *Maribel-bis* and *Maribel-ter* measures which were targeted at firms exposed to international competition (mainly in manufacturing).
36. The difference with other European countries should not be overstated. As noted in the 1997 *Survey*, while in Belgium most jobless persons may remain in the unemployment schemes for an indefinite period, in many other countries after a certain period they are moved either to social assistance or to other social security schemes.
37. It is now possible to run together four contracts, each of a maximum duration of six months.
38. However, the "equal opportunity aspect" is not explicitly covered in the OECD job strategy, while it is one of the four pillars of the EU approach.
39. This Law required the social partners (*Conseil central de l'économie*) to assess at regular intervals the international competitive position of the Belgian economy on the basis of specific criteria – notably the evolution of the export performance and labour costs – and make recommendations to the government which, if competitiveness was threatened, could take a number of measures.
40. For example, airline deregulation in the United States led to a large employment creation in this sector, while electricity deregulation in the United Kingdom led to the opposite result.
41. As a result of the "*franchise sociale*" and the "*franchise fiscale*", for certain persons, co-payments (excluding for pharmaceuticals and care provided in psychiatric establishments) are waived above specific yearly levels. The "*franchise sociale*" covers widows,

disabled, pensioners and orphans (or VIPO) as well as some other groups with specific social benefits – such as old persons with a minimum social assistance benefit. The "*franchise fiscale*" covers taxpayers with especially low revenues.

42. Compulsory public health insurance is provided by two, financially separate, schemes: the general scheme ("*régime général*") covers both major and minor risks and applies to employees and civil servants, retired, disabled and their dependants (representing nearly 90 per cent of the population); the scheme for the self-employed and their dependants ("*régime des indépendants*") – representing around 10 per cent of the population – covers only major risks. Major risks mainly represent in-patient (hospital) care and special (expensive) technical services. Minor risks include out-patient care, medicine, dental care, etc. The compulsory scheme includes both health insurance cover and income support in the event of illness.

43. More precisely, there are currently 87 *mutualités* consolidated in five national unions.

44. According to the authorities, co-payments represented some 8 per cent of the cost of ambulatory and hospital care in 1995. Since pharmaceuticals are not covered by the "*franchise sociale*" and the "*franchise fiscale*", their effective co-payment rate is higher, which brings the overall co-payment rate to around 15 per cent of medical expenditure covered by the compulsory insurance.

45. Also, both the *mutualités* and private insurance companies are forbidden by law from reinsuring the co-payment.

46. Before the Belgian health care system acquired its present institutional structure, to gain financial stability and political protection, *mutualités* used to associate with various fractions of the labour movement, which were largely organised on the basis of philosophical or religious tendencies – *i.e.* catholic, socialist and liberal.

47. Since 1995, however, this pooling approach has been modified somewhat, and in the coming years the *mutualités* will be responsible for a growing proportion of their financial shortfalls (see below).

48. For some care, reimbursement may be less, not exceeding 60 per cent. For some activities, on the other hand, the fee is restricted to the amount reimbursed under the sickness insurance cover.

49. There are also a few for-profit hospitals; these hospitals are not entitled to public subsidies for construction and equipment.

50. The standard cost per day is based on several factors, including historical costs, average costs of a sample of hospitals with similar characteristics, case mix and work load.

51. This is true for private non-profit hospitals but not for private for-profit hospitals which, as noted above, are not entitled to public subsidies.

52. To assure the service of these loans, appropriate interest and repayment charges are included in the budget of hospitals and hence in the daily price of hospital stays paid by the *mutualités*.

53. Throughout this chapter, figures on health care spending are from OECD Health Data 98. They include health spending by the public sector (federal government and regional governments) for current and investment purposes, as well as spending by the private sector (including household spending on over-the-counter drugs). It has nonetheless been claimed that these data may underestimate the true expenditure on health since they omit a number of expenditures, notably by regional authorities.

See Wouters, Spinnewyn and Pacolet (1988). In order, above all, to make comparative studies between countries possible, it would seem that the quality and coverage of health statistics need to be improved and standardised.

54. More precisely, current health expenditure covered by the *Institut national d'assurance maladie-invalidité* (*Régime général and Régime des indépendants*).

55. In an appendix to this report, experts from two universities (*Université libre de Bruxelles* and Catholic University of Louvain) show that, on the basis of a theoretical norm like the one mentioned above, medical expenditure is somewhat greater in Flanders than in Wallonia.

56. The only major exceptions are: for examination on a Magnetic Nuclear Resonance apparatus (MNI), since Belgium has the lowest number of MNIs per capita in Western Europe (3.1 per 1 million inhabitants); and, as in most European countries, for transplants, mainly due to a shortage of donors. Steps have been taken to increase the number of MNIs, as part of a multi-year plan.

57. The "norm" uses as the price deflator the so-called "health index" which excludes the price of tobacco, alcohol, and petrol and diesel fuel.

58. Although this responsibility had already existed since 1963, it was only "theoretical", and cumulated surpluses (*vis-à-vis* the government) and deficits were wiped out in 1974, 1980 and 1988. As part of the new arrangement, cumulated surpluses and deficits since 1980 were frozen, but the deficits would be reinstated if the *mutualité* which recorded them were to return into the red.

59. In the event of the norm being exceeded, the overrun percentage subject to cost participation is capped; if the norm is under-utilised, the percentage is not capped. This can result in a biennial cycle of overruns.

60. Health care is generally regarded as a "superior good". Hence, it is normal, over the long term, for health expenditure to increase faster than GDP.

61. For many people in Belgium, a fundamental difference between the Belgian health system and a co-ordinated care system could lie in the fact that, under the Belgian compulsory insurance scheme, the decision to give a patient expensive treatment is taken by experts working for public bodies or non-profit organisations (*mutualités*), whose concern is to prevent abuses and reduce needless consumption of medical care, whereas under a co-ordinated care system, that decision is taken by employees of commercial insurance companies who could be suspected of seeking to restrict costs by refusing care that is justified.

62. Efforts should be made to computerise medical data systems, not only to make them more efficient from an administrative point of view, but also to facilitate the use of the information generated for relevant research.

63. The study, which was commissioned by the nine ministers with responsibilities in the health area in Belgium, was carried out in 1997 by the *Institut scientifique de la santé publique Louis Pasteur* in co-operation with the *Institut national de statistique* and the *Centre universitaire de Limbourg*.

List of acronyms

ASLK/CGER	Financial company
BATC	Brussels Airport Terminal Company
CPI	Consumer Price Index
EMU	European Monetary Union
EU	European Union
FOREM	Regional placement service
GIMF	*Société régionale d'investissement de Flandre*
HMO	Health Maintenance Organisation
INAMI	*Institut national d'assurance maladie-invalidité*
MINIMEX	Minimum social assistance scheme
MNI	Magnetic nuclear resonance apparatus
NAWRU	Non-Accelerating Wage Rate of Unemployment
ONEM	Federal body administering unemployment benefits
ORBEM	Regional placement service
PHARMANET	Medical data base
R&D	Research and Development
RMMMG	*Revenu minimum mensuel moyen garanti*
RVA/RLW	Public sector airport operator
SMEs	Small and Medium-sized Enterprises
VAT	Value Added Tax
VDAB	Regional placement service
VIPO	*Veuves, invalides, pensionnés et orphelins*

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Annex

Calendar of main economic events

1996

December

Under the law on promoting employment and the preventive safeguarding of competitiveness, the government, on account of the failure of the trade unions and employers' organisations to conclude an inter-professional agreement, sets the maximum nominal increase in per capita wage costs at 6.1 per cent for 1997 and 1998.

1997

February

The European Commission gives its agreement to a new Maribel regulation applicable to all industrial sectors as from 1 July.

April

A Royal Decree gives a legal status to the activity of collective investment undertakings specialising in the acquisition of stakes in companies with high growth potential, listed on the recently created EASDAQ (European Association of Securities Dealers Automated Quotation) and EuroNM (Euro Nouveau Marché) markets.

May

New legislation on public procurement, *i.e.* state orders, to bring it into line with European regulations.

The government decides to liberalise telecommunications markets completely from 1 January 1998.

June

Partial privatisation of Distrigaz.

August

Completion of the privatisation of the Crédit Agricole: the Banque Bacob acquires the last block of shares of the former public credit institution.

September

The federal government approves the contract specifications for a third mobile telephone (GSM) network.

Creation of Telenet-Flandres, a telecommunications network which will compete with the federal telephone company Belgacom.

October

The federal government presents its 1998 budget. The overall budget deficit will be cut from 2.8 per cent of GDP to 2.3 per cent in 1998. At the federal level, fiscal strategy consists in stabilising, as a percentage of GDP, the primary surplus at the 1996 level, *i.e.* 5.3 per cent of GDP (on the basis of the Belgian definition). As regards the budgets of the Regions and Communities, they comply with the recommendations of the Conseil Supérieur des Finances: their net borrowing requirement must fall to 0.4 per cent of GDP.

The National Bank raises its central rate and most of its other rates by 30 basis points. The central rate rises from 3 to 3.3 per cent.

November

The BBL is taken over by the Dutch Banking and Insurance group ING.

1998

January

Merger of the financial institutions KB and CERA, the Assurances Boerenbond Belge (ABB) and the holding company Almanij, to form a Banking and Insurance group.

February

The *Conseil de la concurrence* approves three opinions concerning: the notification of concentrations, the obligation to inform workers about competition matters, and the modification of the framework for the protection of competition.

March

Further to the budget control, the government announces its new public finance targets for 1998: a primary surplus of 6 per cent of GDP, a public deficit of 1.7 per cent (compared with 2.3 per cent in the previous budget), and a total debt down to 118.5 per cent of GDP (compared with 122.3 per cent projected initially).

May

The European Council decides that Belgium (as well as ten other candidate countries) meets the requirements for the adoption of the single currency and will thus join the euro-zone on 1 January 1999.

June

The European Commission gives its go-ahead to the take-over of the Générale de Banque by the Belgian-Dutch group Fortis.

September

A transitional system of compulsory reserves placed with the National Bank comes into force for a period of four months. The rate on end-of-day advances is cut by 5 basis points to 4.5 per cent.

October

The federal government presents its 1999 budget. The overall budget deficit will be cut from 1.6 per cent of GDP in 1998 to 1.3 per cent. Debt will fall from 118 per cent of GDP at end-1998 to 115 per cent at end-1999. The federal primary surplus will be stabilised at 5.3 per cent of GDP. In line with the recommendations of the *Conseil supérieur des finances*, the Communities and Regions will present a balanced budget for 1999. The overall primary surplus will be 6 per cent of GDP.

November

The social partners conclude a preliminary agreement binding the entire private sector for 1999 and 2000. For 1999-2000, wage increases (indexation and seniority included) may not exceed 5.9 per cent. This figure is a benchmark which may be exceeded when a company makes a special effort on the jobs and training front.